



CIHI's Annual Report

2024-2025



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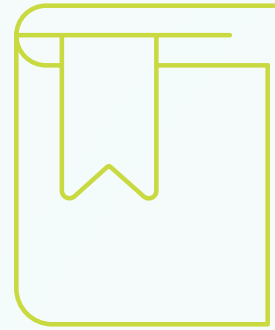


Table of contents

About CIHI	4
Message from CIHI's Board Chair	6
1 year in: Q & A with CIHI's President and CEO, Dr. Anderson Chuck	8
Our accomplishments	12
Our people.....	56
Risk management	72
Leading practices	76
Financial statements	82

About CIHI

Imagine a health system where every Canadian gets the care they need, when and where they need it — powered by trusted, accessible data. That's the future CIHI is building.

The Canadian Institute for Health Information (CIHI) is an independent, not-for-profit organization that provides essential information on Canada's health systems and the health of Canadians.

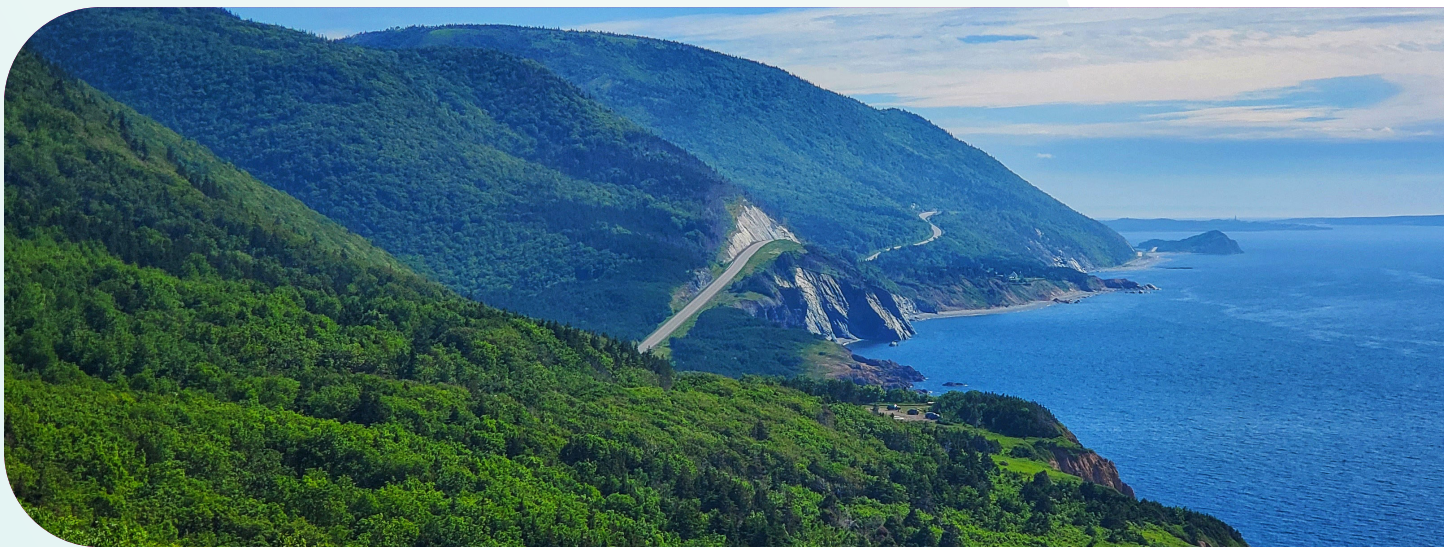
Health information has become one of society's most valuable public goods. It informs policy, management, care and research, leading to better, more equitable health outcomes for all Canadians.

For over 30 years, CIHI has earned the trust of health systems as the main gatherer, packager and disseminator of health information. To succeed in this role, we have evolved to be both knowledge leaders and service providers — in tune with the health systems' needs while setting the pace on data privacy, security, accessibility and innovation.

We're working hard to ensure we remain a vital partner in a rapidly changing world. We are modernizing our tools, embracing digital innovation and trying to meet the growing demand for timely, integrated and actionable insights.

Our team is driven to ensure that health providers have the trusted information they need. That researchers have more opportunities to innovate. That policy-makers can make better decisions. And, most importantly, that patients across Canada get the best care and outcomes.

Better data, better decisions, healthier Canadians isn't just our vision — it's our commitment to you, Canada.



Land acknowledgement

As CIHI works toward better health for all people in Canada, we acknowledge that we live and work on the traditional territories of First Nations, Inuit and Métis Peoples. Our work is grounded in cultural safety and humility, respectful engagement, and Indigenous-driven processes and partnerships.

Message from CIHI's Board Chair



Few organizations are as essential to the digital transformation of Canada's health care systems as CIHI. Throughout 2024–2025, I've been privileged to continue working with the Board and leadership team in shaping its direction.

As we collectively embrace the potential of connected care to improve health experiences and outcomes for everyone living in Canada, CIHI assumes the unique and necessary roles of both managing data modernization and serving as the trusted, independent broker that brings all stakeholders together.

In 2024, we welcomed our new CEO, Dr. Anderson Chuck, who previously helped make tremendous shifts in Alberta's strategic use of health care information in his role as chief health economist at Alberta Health Services. He is an energizing leader who now helps ensure that CIHI continues to provide meaningful and actionable insights to all those who rely on our organization. Dr. Chuck will carry forward the great momentum built by his predecessor, David O'Toole, whom we thank for 10 years of dedication to this mission.

In this report, I encourage you to read about CIHI's significant accomplishments over the past year — which, by their nature, are impactful to Canada as a whole. The team developed a world-first data standard that will be the bedrock of how patient data will soon flow consistently, accurately and interoperably to providers across health systems.

We've now created more than 20 indicators in support of the country-wide Shared Health Priorities initiative, which focus on accessing primary care, planning for health workforce and surgeries, accessing mental health and substance use services, modernizing electronic health information and helping older Canadians age with dignity.

Early this year, CIHI released a new standard for health workforce information, which will open an expansive window for decision-makers to understand the current and future state of workers in 8 separate health professions.

This data innovation is what makes CIHI so essential. In undertaking our mission, we are careful to ensure that no advance is made without the input of many voices. To that end, we are building collaborations with all impacted stakeholders, including the public and members of Indigenous communities.

Thank you to everyone, within and beyond CIHI, who has helped propel this transformation. In our person-centred system, we are in this together.

Dr. Vivek Goel
Board Chair



1 year in: Q & A with CIHI's President and CEO, Dr. Anderson Chuck

In summer 2024, we welcomed Dr. Anderson Chuck as CIHI's new president and CEO. He arrived from Alberta Health Services where, as chief health economist, he helped shape the province's strategy for a sustainable health system transformation that champions value-based care.

What draws you toward the service of health care — and, ultimately, CIHI?

I was always drawn to health care: my father was a physician and I aspired to be the same. While in university, I interned at a facility devoted to Alzheimer's care, and I was fascinated and humbled by what it actually took to deliver care — the clinical (doctors and nurses) and non-clinical (procurement, finance, data information systems) components that needed to come together and work as a team in order to provide high-quality care.

That inspired me to study public health and health economics — to seek the optimization that gets the most out of scarce resources to maximize outcomes for patients. It sparked a passion for how data is the fuel to inform decisions that improve the patient experience and impact the lives of Canadians. Coming to CIHI was an incredible opportunity to do this work on a broader scale.

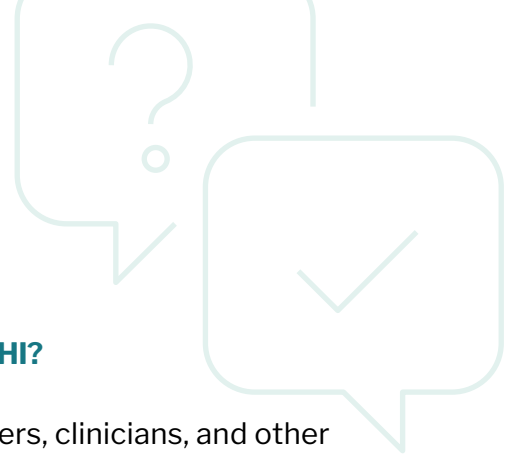
You've stepped into this role at a pivotal time for health care in Canada.

What's your perspective on the moment we're in and the opportunity ahead?

It's an exciting time because now, more than ever, we have the case for change. Now is the moment to act. Across the country, there's a growing urgency to fix what's not working in our health systems — but that urgency also creates opportunities for change.

Canada is sitting on one of the richest sets of health data in the world. If we can harness it — across jurisdictions in partnership with patients, providers, researchers, innovators and governments — together we can shape a health data ecosystem that puts patients first and ensures our data sovereignty and security while supporting Canadian innovations in novel therapies. Ultimately, this will help improve how decisions are made and how care is delivered, and improve the care for patients in Canada.

This is a chance for Canada to lead and for CIHI to play a meaningful role in shaping a better, more collaborative future for health care.



What were your initial priorities when you joined CIHI?

I set out to learn as much as I could. I met researchers, clinicians, and other data users and stakeholders who gave both inspiring and humbling feedback. It was clear that CIHI has deep value in providing clarity around how to improve health systems — but also that system needs are changing.

It helped crystallize that our responsibility at CIHI is to adapt to meet the ever-evolving, changing needs of our users. One was around the need for timely information — a clear call to find ways to make near-real-time data available. Another was expanding to fill gaps in data across health systems, with primary care top of mind.

In my first few months, I came away with a strong feeling that, at CIHI, we are privileged to be in a position of trust and honoured to help serve Canadians across the country.

CIHI is in the midst of a transformation. What does that mean to you?

In our case, I believe transforming means evolving in step with the needs of those who count on us to access the data they need when they need it. As we expand this positive impact on our stakeholders, we must also never lose sight of the context behind the data.

I think the world is evolving faster than ever. New technologies, like AI, mean there are more opportunities for good data to have a big impact than ever before. But we have to navigate these opportunities with Canadians' best interests in mind. What data do we need for the coming years? How can we drive toward better health for all? How do we equip innovators and researchers with the data that can drive change?

Across our widespread, diverse country, how do we foster greater equity in health systems?

It takes an ongoing, focused commitment to improve the health of all Canadians, including vulnerable or marginalized populations. That is the promise of Canada's universal health care, and that promise carries responsibilities for organizations such as CIHI.

The foundation here is trust. Trust is earned over time by respecting the self-determination of populations such as First Nations, Métis and Inuit communities. For CIHI, it's demonstrating that we are here to listen, learn and help them make progress on what is important to them that they measure in terms of health. Not only what we should be collecting, but also how we should do so and how it should be interpreted.

We know there have long been underserved populations in Canada. To help build equity, we will ensure they direct us on how we can develop their information — not only engaging them, but asking them to pave the way forward.

Looking ahead, what do you see?

Data is the fuel that drives change in our health systems. In this transformation, which includes the intentional and responsible use of AI, CIHI will help establish a seamless health data ecosystem capable of producing the kind of insights that can help solve the challenges and gaps in the delivery of care in Canada.

The vision is that when a patient receives health services, they will genuinely expect and receive the best care.



Our accomplishments

Learn about what we accomplished in these areas:

- ✓ Health workforce
- ✓ Shared Health Priorities
- ✓ Hospital data transformation
- ✓ Pharmaceuticals Data and Information Roadmap
- ✓ Equity stratifiers
- ✓ Indigenous health
- ✓ Connected care
- ✓ Data stewardship
- ✓ Artificial intelligence

Health workforce



Measuring our current and future health workforce: CIHI data essential to plans to broaden Canadians' access to care

Here's the bird's-eye view: across the country, health care workers are under pressure — facing high patient volumes, record-level burnout and absenteeism, and a system that urgently needs reinforcements. Canada's ability to expand access to care depends on understanding not only the size of our workforce, but also how it's changing.

In primary care, those needs are starkly apparent. In 2023, the [CMAJ estimated that 6.5 million Canadians](#) didn't have a family doctor. In 2025, CIHI found that 17% of Canadian adults can't access a regular care provider — a figure that climbs to 26% for those age 18 to 34.

We are now seeing an expanding network of health care professionals who can act as first points of contact for patients. It's vital that we understand such evolving health workforce dynamics, and over the past year CIHI has made great strides toward transforming which data can be collected and analyzed — so that the bird's-eye view can become granular.



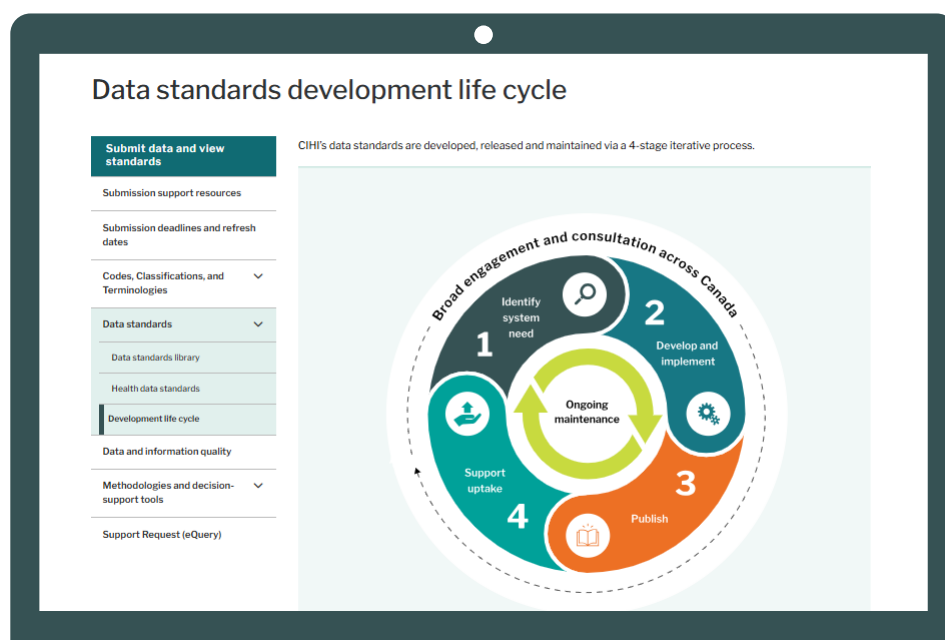
Transforming a data standard so we can do more

In 2022, we released a new, modernized standard for health workforce information that fosters comparable, high-quality, linkable data across various provider groups. For certain professionals, CIHI has been able to collect detailed data — but for others, we've relied on aggregate data.

The goal is to be as precise as possible to fill knowledge gaps and best support pan-Canadian health workforce planning, modelling and policy development. In addition to the data CIHI collects and reports about physicians, we are collaborating with regulatory bodies to receive data about

- Licensed practical nurses;
- Registered nurses;
- Nurse practitioners;
- Registered psychiatric nurses;
- Occupational therapists;
- Physiotherapists;
- Pharmacists; and
- Personal support workers.

We are helping each regulatory body transition to the new data standard and embed it in their respective systems. CIHI has also worked with Health Workforce Canada to engage new priority professionals — midwives, psychologists, social workers and paramedics — to likewise adopt the standard and enhance the available information on these key workforces.

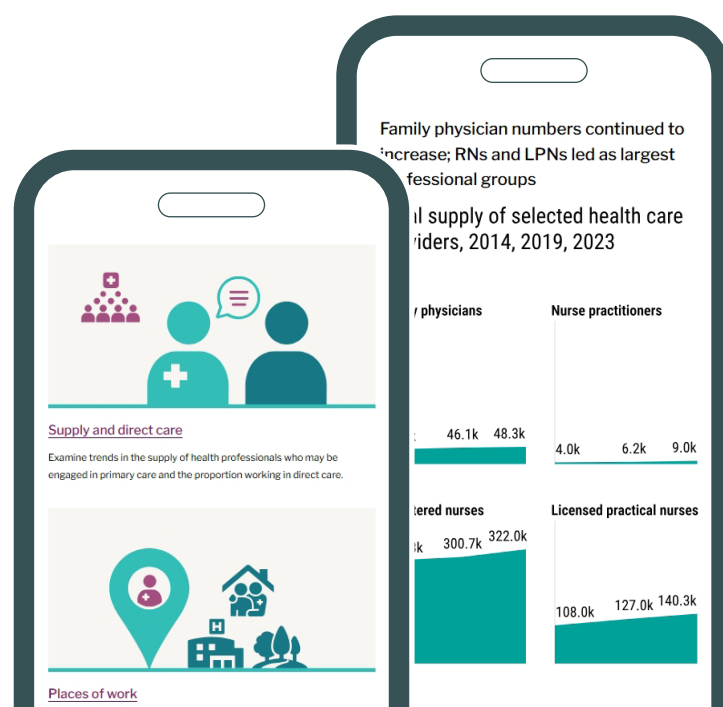


State of health workforce findings released

In December 2024, we released our annual report on the [state of the health workforce](#), which focused on a range of health care professionals working in primary care and in the community. It revealed the following:

- The overall community-based health workforce is growing in Canada, with 3 provider groups particularly represented. The proportion of physiotherapists working in community settings rose by over 15 percentage points in the past 10 years. And nearly three-quarters of pharmacists now work in community settings, as do one-third of nurse practitioners.
- The network of primary care providers is expanding. Growth was led by pharmacists and physiotherapists — 73% and 70%, respectively, of them work in community settings such as primary care. We also found that nearly 56,000 registered and licensed practical nurses work in community settings.
- On average, family physicians have seen 18% fewer unique patients over the last 10 years. Many factors are believed to play a role, including older patients and patients with increasingly complex care needs, who take longer to see; the evolution of new models of care; administrative burden; and policy changes.
- In 2023, many jurisdictions continued to expand prescribing authority for minor ailments to nurses and pharmacists, and also opened nurse practitioner-led clinics in areas with limited numbers of family doctors.

Ultimately, CIHI's unique analysis — which will expand in scope as our data standard is adopted across Canada — helps system leaders understand the state of more and more key professions in order to make informed decisions around enhancing equitable access to care.



Understanding health workforce vacancies

Essential to understanding the state of health care professions is knowing what roles are going unfilled. To do this, we must measure job vacancies — an effort complicated by the fact that there is no agreed-upon way to characterize these unfilled positions in a standard way. This knowledge gap must be filled to ensure accurate workforce data. So in 2024, our team performed an [analysis designed to understand how vacancies are defined](#) and reported across Canada.

As most ministries of health have or are developing a definition of vacancy, CIHI is collaborating to bring clarity and consistency on the subject. In October 2024, we led a workshop at Health Workforce Canada's first symposium to examine this issue and learn what our government partners felt was most important as we first **define** what we will then **measure**.

This ongoing effort will ensure that we standardize an approach so that data can illuminate a pan-Canadian picture. Our first vacancy measure is expected in fall 2025.



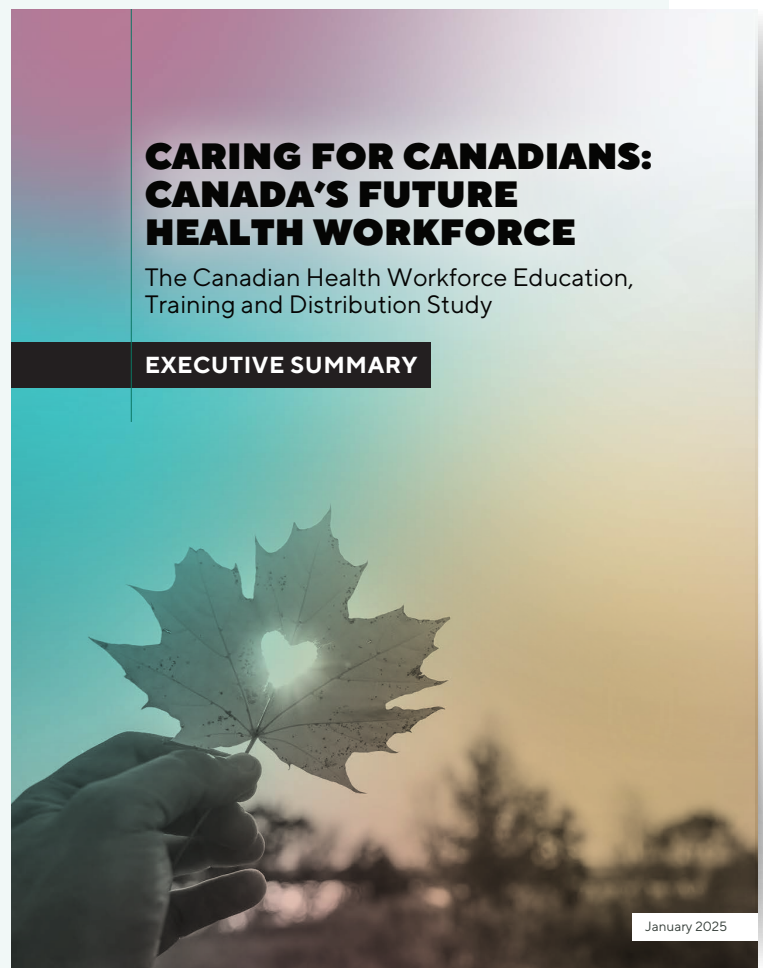
CIHI data key to new study on future health workforce

CIHI data and tools were vital for the first pan-Canadian study to look at the education, training and distribution of health care professionals in a bid to predict our upcoming workforce.

Designed to support federal, provincial and territorial governments, the study confirms that our workforce doesn't currently meet the needs of Canadians — and that this will only get worse over the next 10 years.

It predicts that tens of thousands of practitioners will be needed in the years to come, from physicians to registered and specialized nurses, from occupational therapists to pharmacists. Its call to action is clear: governments must act now by forging new collaborations, supporting a strong data foundation, and building a unified approach to health workforce planning and education.

[Read the summary of the Canadian Health Workforce Education, Training and Distribution Study](#)





Shared Health Priorities

Tracking progress where it matters most: CIHI plays pivotal role in measuring Canada's big health priorities

Canada's health systems are facing urgent pressures. In the face of these challenges, CIHI has worked closely with federal, provincial and territorial governments and other stakeholders to measure improvements in [shared health priorities](#) — critical focus areas where progress can directly improve care for Canadians. These priority areas are

- Primary health care;
- Health workforce and surgeries;
- Mental health and substance use;
- Electronic health information; and
- Aging with dignity.

Our government partners have committed to collaborating with CIHI to track progress in these priority areas. We've taken a multidisciplinary and diverse approach to this effort, purposefully engaging a broad range of partners, including the Shared Health Priorities Advisory Council; data partners from Statistics Canada, Canada Health Infoway and Integrated Youth Services; system and measurement experts; and people with lived experience.

The first step to ensuring we measure the most meaningful aspects of care is to establish common definitions that can be used across Canada. As of early 2025, this has led to a new set of more than 20 indicators in the original priority areas — equipping system leaders to build precise strategies to meet their communities' unique needs and providing a pan-Canadian perspective on these critical shared health priority areas.

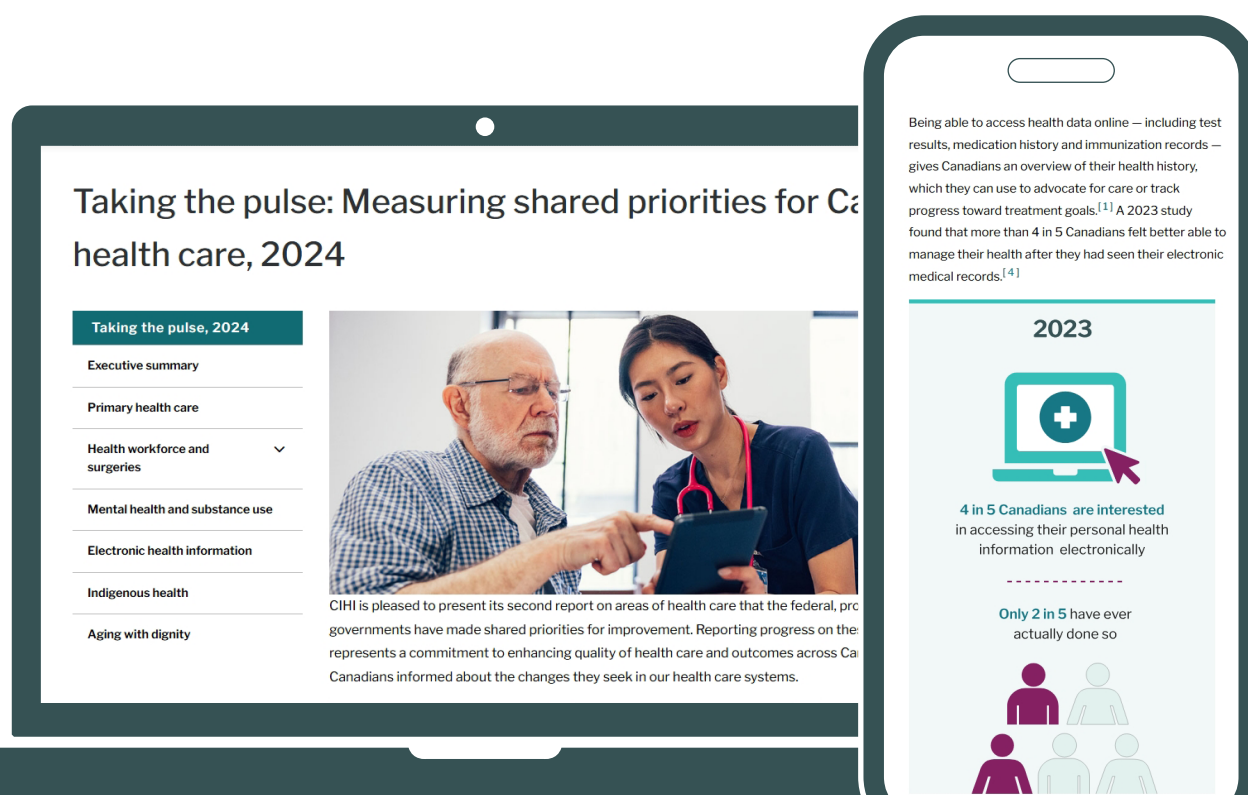


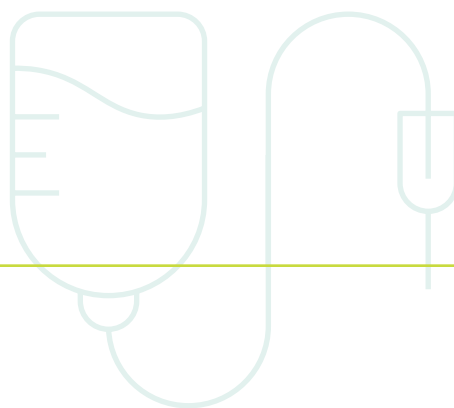
Taking the pulse on Canadian health priorities

In October 2024, we released our second [Taking the pulse report](#), which provides a bird's-eye view of 4 of the priority areas. The goal is to keep Canadians informed on these important health topics while establishing baseline results to track improvements over time.

This report garnered national media attention. Highlights include the following:

- **Primary health care:** 83% of Canadian adults have access to a regular health care provider. The remaining 17% — more than 5 million people — struggle to access a regular care provider, a figure that increases to 26% for those age 18 to 34. Just one result of this is that people without access to a regular provider are less likely to find care in their communities and more likely to make emergency department visits.





- **Health workforce and surgeries:** In a 1-year span, nearly all provinces and territories had more family physicians, nurses and nurse practitioners on the job. Surgery numbers also rose, rebounding to pre-pandemic levels. In fact, more than 2.3 million surgeries were performed in 2023–2024, which was 5% higher than in 2019–2020. For adults 65+, that increase was 14%.
- **Mental health and substance use:** In 2023–2024, half of Canadians waited 25 days or less for community mental health counselling — but 1 in 10 waited 5 months or more. In 2023–2024, Canada had 92 Integrated Youth Services sites that provide young people with community-based services for mental health, substance use and other needs under one roof, up from 63 sites the year before.
- **Electronic health information:** While 81% of Canadians want to access their health information online, only 2 in 5 (39%) have done so — and only 30% of younger adults age 18 to 24.

Along with physical health challenges, I was dealing with mental health issues that were overwhelming. After my GP retired, I've struggled to find a new family doctor, so I've had to manage my own health care as best I can.

Jenna Kedy, Patient Partner, who lives with several chronic ailments, including rheumatoid arthritis. When she transitioned out of pediatric care, her family doctor also retired, and she must rely on emergency departments and her university's clinics for support.





Ensuring older adults live with autonomy and respect

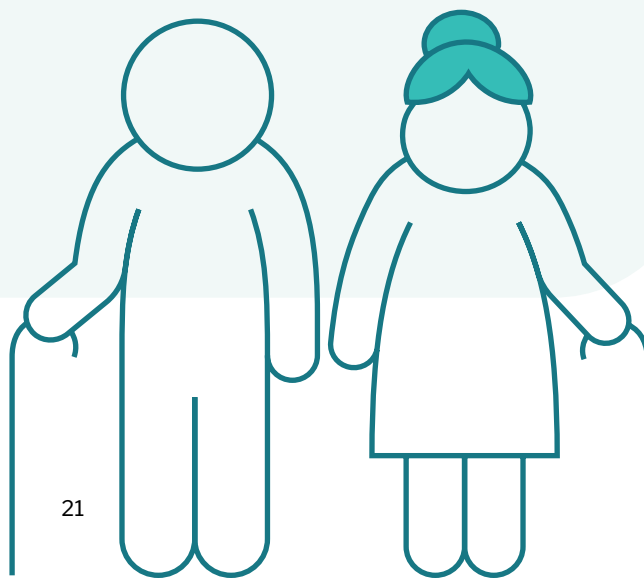
In fall 2024, CIHI announced that a new set of indicators about aging with dignity had been selected and endorsed by the federal, provincial and territorial governments.

These indicators will enable a closer understanding of the long-term care (LTC) workforce and its resilience; safety and quality improvement in LTC; and experiences and outcomes affecting LTC residents and their families.

This milestone sets a common foundation for monitoring this area and keeping Canadians up to date on the care that older people need, where they face gaps in services and what needs to change to better support them as they age.

The indicators measure

- Falls in the last 30 days in LTC;
- Potentially inappropriate use of antipsychotics in LTC;
- The proportion of LTC residents who die on the day of transfer to hospital;
- Residents actively engaged in their daily life and care activities;
- Overtime hours worked providing care in LTC; and
- The supply of personal support workers.



Hospital data transformation



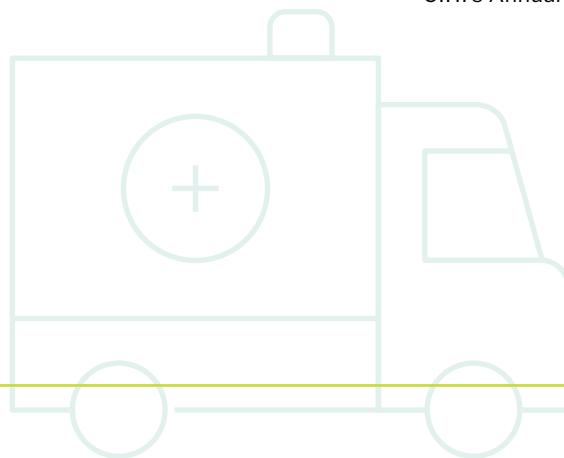
Transforming hospital data: Huge initiative underway to bring new data access

Data from hospitals across Canada has long been CIHI's most complete source of information for health care leaders. Yet it's always been retrospective in nature, available months later because it comes from different systems in different forms and must be manually reviewed, abstracted and coded by health information management professionals — coders — to ensure high quality.

That is about to change. In a far-reaching new initiative, CIHI is working with provinces and territories to make hospital data available faster than ever before. This will empower decision-makers to see near-real-time trends and make decisions that best support patients and providers.

Our team is leading a data modernization project that will mean information from a hospital — including the emergency department — can be received, processed and accessed with little delay. The goal: potential for daily refreshes of hospital data that can address crucial questions about hospital care delivery.





The impact of near-real-time information

Such efficiency will provide institution and government leaders with a timely pulse-check on the state of hospital care. It will mean access to richer data to be able to analyze a broader scope of topics, including any urgent or emergent issues. Having a line of sight into emergency department trends is profoundly important for public health officials across Canada. Knowing how and where patients access care is key to improving their experience with their health system. Data on why people visit emergency departments can reveal how well primary care is holding up in any given region, as well as point to the need for more care to be delivered in the community. Leaders will be able to understand bed availabilities and respond quickly to free up spaces where needed.

Transformations start with collaboration. As the country's data aggregator, we are meeting with hospitals and health authorities to gauge the breadth of near-real-time data available now. CIHI is building a system capable of receiving new data streams and will integrate our 2 existing databases that capture hospital discharge and ambulatory care data into a single system to serve as a reservoir of timely, accurate hospital information.

Meanwhile, CIHI is facilitating and promoting use of AI-driven solutions that will standardize how clinical documentation is turned into structured data. These solutions will automate what has long been a manual process by coders, using technology that scans clinical notes, extracts the right information to chart and codes it to be structured.

A sustainable evolution begins

This level of transformation is complex yet critically needed. As hospitals digitize their own processes, the potential of AI comes into focus. Over the next 3 to 5 years, CIHI will lead the effort to enable a seamless flow of pan-Canadian hospital data, focusing on minimizing additional burden on our hospital partners. In the near future, a clear pan-Canadian window into what's happening in our hospitals will open permanently.

Near-real-time hospital data for Canada

CIHI is working with several provinces and territories to source near-real-time information on hospital admissions and discharges, either directly from hospital information systems or from provincial aggregators. With these new information flows, CIHI can produce up to daily refreshes of statistics on emergency departments and hospital admissions across the country, which will substantially enhance Canada's response to urgent and emergent public health issues.

This initiative will leverage the various digital health innovations and implementations happening in the provinces and territories, apply the “collect once, use many times” principle to concretely reduce redundancy, enhance data timeliness and facilitate data sharing across Canada. Like everything that CIHI does, collaboration and partnerships with both the public and private sectors will be foundational to this transformation.



Auto-coding project in B.C.

CIHI is partnering with the B.C. Ministry of Health to support the automation of hospital data coding.

The province has made great strides in auto-coding data for its provincial cancer registry and is keen to bring the learnings to its emergency departments.

This initiative will significantly boost the coverage and richness of emergency department data in the province.

As well, it may inform a direction that CIHI can use to work with all provinces and territories toward new auto-coding solutions.



Pharmaceuticals Data and Information Roadmap



“All Drugs, All People”: On the use, safety, effectiveness and cost of Canada’s prescription drugs

Against a complex backdrop, CIHI is striving to build a more complete data set for an area that has long been challenging and whose impact is extensive. Namely, understanding how prescription drugs are used, as well as their safety and effectiveness.

Better information will enable decision-making around access to and use of pharmaceuticals across Canada.

In January 2025, CIHI announced a 3-year agreement with Canada’s Drug Agency (CDA-AMC) to build a foundation for pan-Canadian drug data and analytics. It marks an important step in our ongoing work on our roadmap designed to support decision-makers.

Through the CDA-AMC partnership, CIHI is now accelerating efforts to

- Advance data collection by acquiring more complete drug data;
- Make data accessible for analyses; and
- Modernize our pharmaceutical data holdings.

Transforming our knowledge of prescription drug use in Canada requires deep collaboration. CIHI is actively meeting with provinces and territories to gauge what is most important to them in this area and how we can work together to serve our collective interests.



Leading a coordinated effort to understand prescription drug use

CIHI's efforts in drug data have long been grounded in our National Prescription Drug Utilization Information System (NPDUIS). With this data, we can compare the use of publicly funded (and some privately funded) medications across Canada. As part of the roadmap, we're working with government stakeholders to receive additional drug data, and we're streamlining internal NPDUIS processes so existing data can be made available more quickly to address system questions.

To let users explore NPDUIS data right now, our team released a tool that allows people to interact with the information and explore data visualizations on the most popular topics, such as drug expenditures and drug use among older Canadians.

CIHI's drug data modernization is also a key part of the transformative efforts underway in the areas of hospital data and connected care — 2 other themes explored in this report.

And, with the launch of Canada's new pharmacare program in October 2024, we're working with Health Data Research Network Canada to analyze data for CDA-AMC around pharmacare's initial priority areas of diabetes and contraceptives.



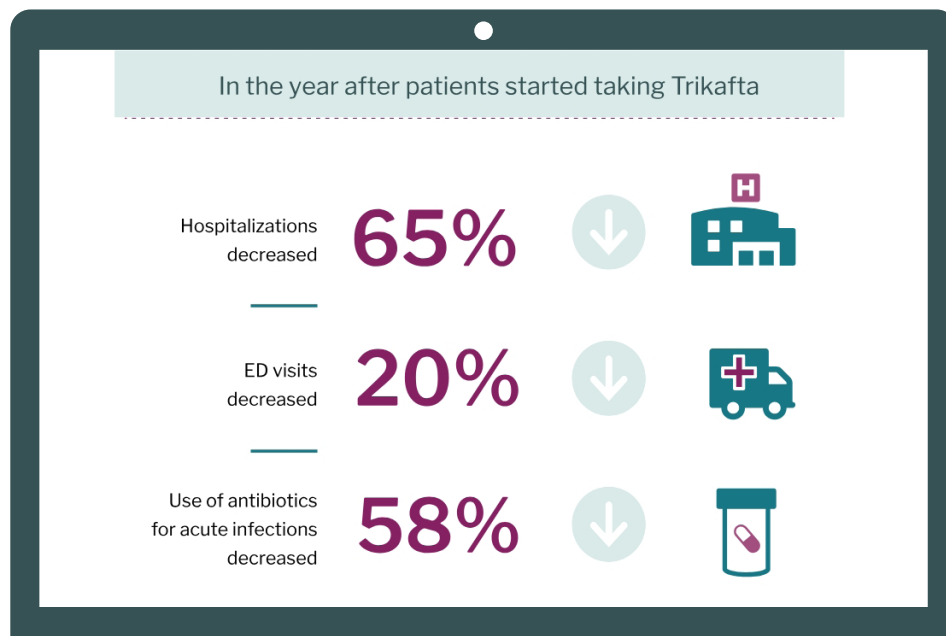
Update: Drugs for rare diseases

CIHI continues to be a key player as the Government of Canada advances its National Strategy for Drugs for Rare Diseases.

Our September 2024 [report on rare disease analyses](#) describes key considerations when using CIHI data to analyze rare diseases in Canada — as well as opportunities to better capture diagnoses, prescription use and care experiences in various settings. We aim to use data to better identify rare diseases, understand people's health system journeys and track patient outcomes.

In November 2024, CIHI released analyses related to 2 such diseases:

- Patients with [cystic fibrosis](#) who started taking Trikafta — the most common, albeit expensive, drug in its class — had fewer hospitalizations, emergency department visits and antibiotics than a year earlier.
- For people with [Huntington disease](#) who live in long-term care facilities, we quantified some of the difficult symptoms they experience and measured the resulting use of medications to treat them in the absence of a treatment for Huntington disease itself.



Equity stratifiers



Toward the promise of equitable care: Dissecting data to identify precisely who has unmet care needs

Canada's health care systems promise equity, yet measuring the experiences of Canadians has long been tied to averages — giving policy-makers an incomplete understanding of health inequities.

CIHI is looking to complement what we learn from averages with true equity analysis and reporting. Across all settings, from hospitals to primary care — and all points in between — the goal is to bring clarity to the experiences of subpopulations of Canadians.

Through its work, CIHI is able to dissect broad population data in an ongoing bid to better understand how variables such as geography, income level, education levels, sex and gender influence people's health across the country.



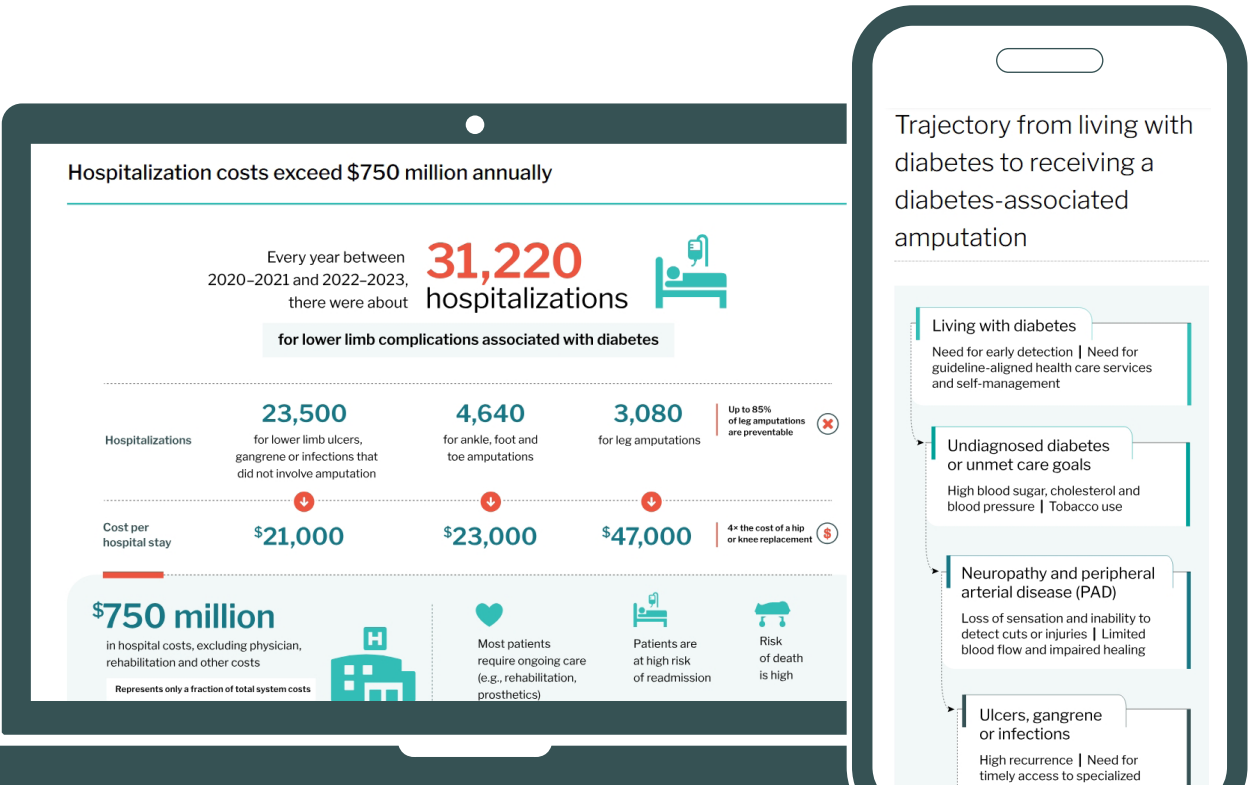
Equity in action: Diabetes outcomes

In fall 2024, CIHI released a report on [equity in care for diabetes](#), a common condition for which patient outcomes have long been tied to inequities. We focused on lower limb amputations, which we know can serve as a measure of someone’s access to quality diabetes care.

The report demonstrated that some Canadians face disproportionate risk of amputation. Lower limb complications were more common among adults who lived in neighbourhoods with lower incomes, lower high school completion rates and greater social isolation, and in rural and remote communities.

This project illuminates which Canadians may have unmet care needs when it comes to diabetes management — subpopulations of people who may have less access to resources, care and education about living well with diabetes. CIHI also assembled guidance documents on how health systems can proactively advance equity in this important area.

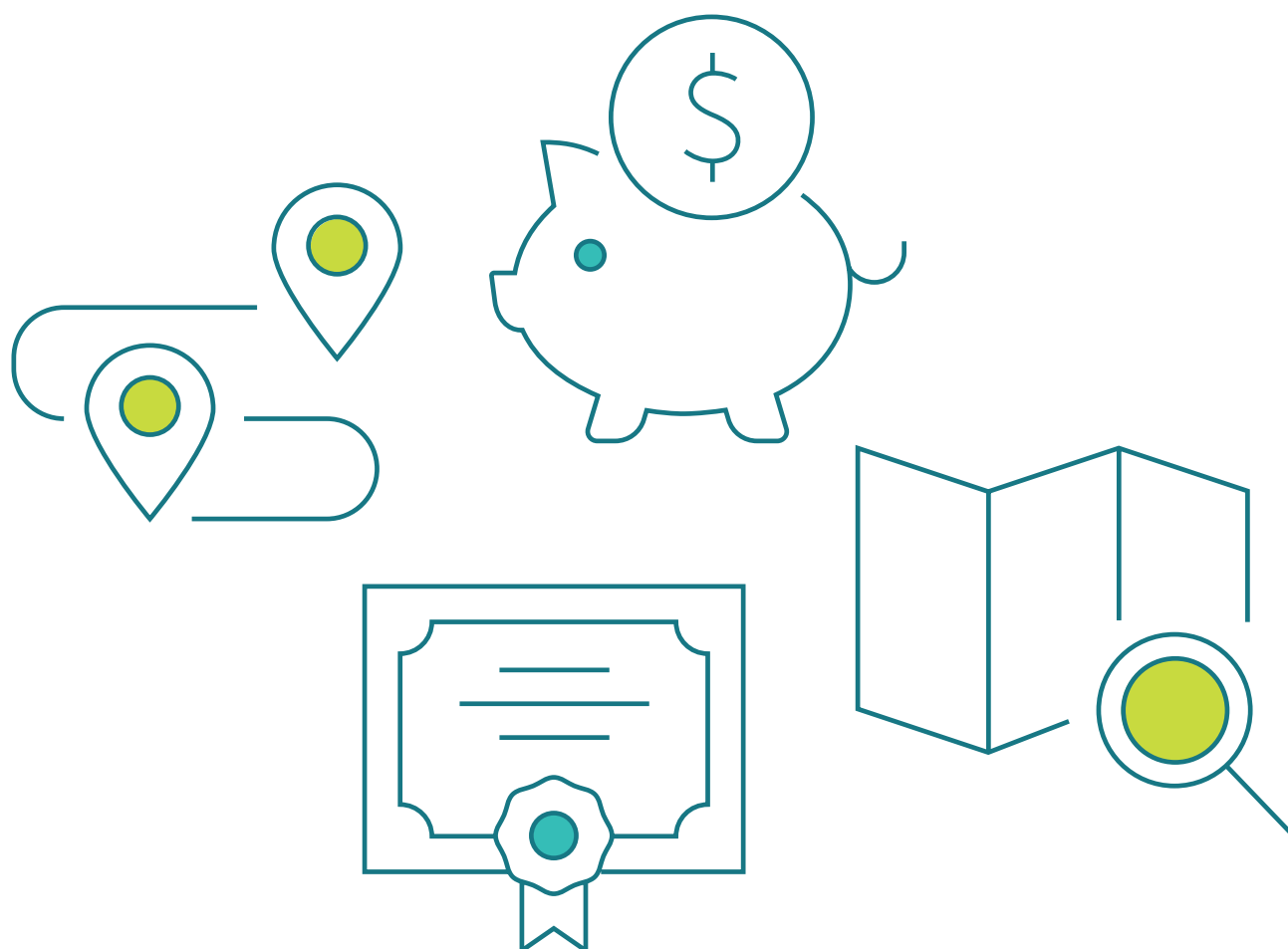
By identifying those at higher risk of poor diabetes outcomes, our analyses provide tools that enable real-world action to reduce inequities.



Supporting system partners through equity stratifiers

Across CIHI, we are exploring ways to better report on equity with health system data that does not routinely include all relevant socio-demographic variables. For example, we can use postal codes to identify areas by level of income and education, and we can apply a remoteness index that can illustrate the potential health consequences of distance from services and resources.

To elevate our health care systems toward greater equity, CIHI is playing an important role in exploring data points, assembling toolkits, building proofs of concept and enabling our partners to make more precise decisions to support the health of all Canadians.





Indigenous health

New partnerships mark Indigenous health priority progress: CIHI's approach to learning and understanding key to building trust

We are continuously learning how to support truth and reconciliation in a safe and respectful way.

As part of CIHI's ongoing commitment to reconciliation, we learn from First Nations, Inuit and Métis individuals, communities and organizations about how to foster a culturally safe health system that respects their right to self-determination. To that end, we seek to work with Indigenous communities, and national and regional organizations that represent them, and to deliver the support they need when it comes to health information.

We are deeply committed to working in collaboration with First Nations, Inuit and Métis Peoples, communities, governments and organizations on health, wellness and data priorities, as defined by them. We are grateful to all Indigenous partners for their confidence in us and our work to support them. In 2024–2025, we expanded our relationships with Indigenous partners, resulting in 3 new memorandums of understanding (MOUs) that continue to provide the foundation for change.



The Métis National Council (MNC) and CIHI formalized their partnership on February 7, 2025, with the signing of a memorandum of understanding to uplift the health data and information priorities of the MNC and Métis-led solutions for better health outcomes.



Christi Belcourt, Reverence for Life (detail), 2013, acrylic on canvas, collection of the Wabano Centre for Aboriginal Health.

Building trust

Over the years, CIHI has come to appreciate that building trust begins with empathy and humility. CIHI has made great efforts in building internal awareness around First Nations, Inuit and Métis history and how colonialism has deeply impacted Indigenous health and lives. By demonstrating an approach that is rooted in respect and humility, and recognizing the sovereignty of First Nations, Inuit and Métis Peoples when it comes to their health data, we begin to earn the trust needed to form long-lasting and fruitful partnerships with Indigenous communities.

As we continue to formalize these relationships and plans with working documents and MOUs, we hold ourselves accountable to co-creating crucial health data solutions and services that are both for and by First Nations, Inuit and Métis partners.

The ultimate goal is for no First Nations, Inuit or Métis individual to be left behind by our health systems. How we get there is by taking the time to understand their priorities and knowing, with humility, that as a non-Indigenous organization, we are approaching this extremely important work from a trust-deficit position. This respectful approach underscores that we are here to follow their priorities, not our own.

Dr. Joseph Emmanuel Amuah, Manager, Indigenous Health, CIHI



How data can lead to change

In order to foster culturally safe health systems, we need data. We are working closely with First Nations, Inuit and Métis thought leaders to identify a set of Cultural Safety and Anti-Racism indicators that are relevant to First Nations, Inuit and Métis Peoples and communities. These indicators are stepping stones on a pathway toward more culturally safe health care, leading to better health outcomes and experiences for First Nations, Inuit and Métis Peoples. Through this work, CIHI aims to zero in on what health and wellness service gaps are faced by First Nations, Inuit and Métis Peoples and co-create the tools necessary to hold health systems accountable and address racism in health care.

These focused insights are not yet achievable with our broader-scale data measures. It is only through these Indigenous partnerships that we can identify specific users of the system, find ways to measure their experiences, co-create respectful data governance mechanisms to share findings and enable leaders to close the gaps they face at the community level.

Our collaboration with CIHI is vital to advancing self-determined health care systems for Métis citizens and communities. By continuing to strengthen our National Data Strategy, we are ensuring that Métis governments have access to accurate, culturally relevant health information.

Our partnership was forged in a culturally safe and beautiful way by bridging our traditional knowledge with our contemporary health care needs. This partnership is a living bridge to matriarchal ways of health — honouring traditional medicines, intergenerational ways of doctoring and being, and Métis ways of life. We are honoured to have an ongoing partnership with CIHI, formally represented through our MOU, as we work toward a future where Métis citizens thrive — grounded in culture, self-determination and a health care system that truly reflects their needs.



Victoria Pruden, President, Métis National Council

A look at recent partnerships

Since November 2024, we've signed MOUs with 3 Indigenous authorities that outline the purpose, principles and priorities that will guide strategies and collaborative efforts. In them, CIHI commits to supporting these organizations with tailored health data and to sharing expertise and insight to help them build their information governance capacity.

Giiwednong Health Link

The not-for-profit [Giiwednong Health Link \(GHL\)](#) delivers e-health tools, resources and training services to 14 First Nations on Manitoulin Island and the north shore of Lake Huron. It seeks to improve the health and well-being of members of its communities through digital health solutions that can integrate electronic medical records and produce meaningful analytics.

Métis National Council

The [Métis National Council \(MNC\)](#) is a government collaborative that represents the Métis Nation whose members are the democratically elected leadership of the Métis Nation. The MNC's primary mission lays in advocacy and in coordinating policy and research, participating in such areas as Parliamentary committees and federal budget submissions.

Sioux Lookout First Nations Health Authority

The 35-year-old [Sioux Lookout First Nations Health Authority \(SLFNHA\)](#) follows recommendations of Sioux Lookout-area Chiefs in supporting the well-being of members of 33 First Nations communities in the region. Its culturally appropriate and community-owned services include an independently governed public health system that uses health information to assess the health of the population.



Working alongside our partner, CIHI, we have rolled up our sleeves to plant the seeds of indicator measurement for racism in health. Through the work of the Cultural Safety Measurement Collaborative, organized by CIHI and led by Indigenous women from across Turtle Island, we have created the first attempt at reducing harm to Indigenous Peoples through measuring racism. We have been involved in the development of many frameworks in Canada and truly feel honoured to be a part of this history-changing collaborative that was supported by Matriarchs, Elders and health leaders while remaining grounded in culture. This shared work is an example of knowing when Indigenous guidance is needed and when to count on partners, allies and dedicated professionals and let CIHI bring the technical knowledge. The Cultural Safety Measurement Collaborative was supported in the beauty and safety of Indigenous culture!



Marion Crowe, Chief Executive Officer, First Nations Health Managers Association

Connected care



Enabling connected care across Canada: CIHI is leading efforts to advance connected care through clear and consistent data standards

Canada's health care systems are under enormous strain. To help ease the burden, CIHI has been working closely with federal, provincial and territorial partners and Canada Health Infoway to address challenges related to key areas identified in the 2023 [Working Together to Improve Health Care for Canadians Plan](#).

In 2024–2025, CIHI made major strides toward one of our biggest priorities: advancing connected care through clear and consistent data content standards. The overall aim of this work is to improve the standardization of Canada's health data to ensure that the best-quality information is available across health systems, particularly for clinical decision-making and to improve patient safety.

Ultimately, we are setting the foundation for the seamless exchange of **consistent, accurate and interoperable** patient health data across health care systems.



Introducing new data content standards for interoperability

CIHI released the first version of the connected care products for open public review in September 2024 — a significant milestone in Canada's connected care journey. This release was the result of many months of collaboration with patients, clinicians, researchers, jurisdictions, vendors, data architects and digital health industry partners. The Pan-Canadian Health Data Content Framework (PCHDCF) is a person-centric data content standard that serves as a library of data elements with value sets, data definitions and a common data architecture.

At the heart of interoperability is a subset of the PCHDCF — the Canadian Core Data for Interoperability (CACDI), representing a first for Canada and leveraging best practices from countries on similar connected care journeys such as Australia and the United States. This data content standard is essential for semantic interoperability, which ensures that the meaning of data is preserved when it is shared. The CACDI defines a standardized set of health data that is essential to coordinated and safe patient care to support interoperability and data exchange across different systems.

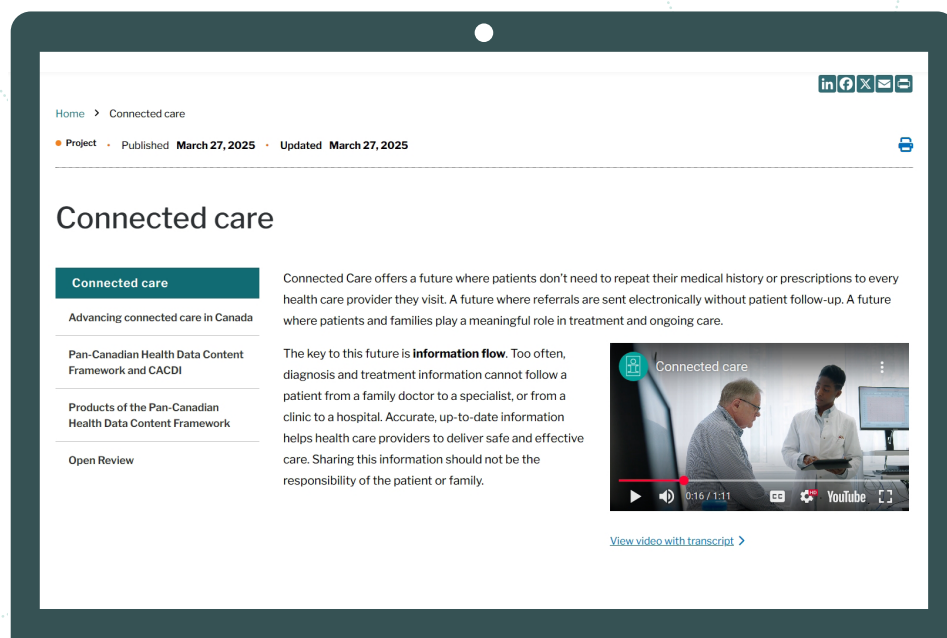
CIHI conducts open reviews with every draft release to invite all stakeholders, including the public, to get involved in the co-design of these products and standards, provide their feedback and share their expertise. This unique model brings maximum transparency to our stakeholders while giving the global digital health community a glimpse at our groundbreaking work in Canada. There is already strong momentum and excitement among health care and digital health professionals across Canada's health care systems, signalling early interest in this initiative.



What's next

It has been a huge year of growth and learning for CIHI and our system partners. Following our successful first release, we will be expanding content in priority areas, working closely with Canada Health Infoway as it incorporates CIHI's data content standards and data architecture into its CA Core+ data exchange standards and harmonizes previously existing data exchange profiles.

Interoperability is a team sport! To achieve connected care in Canada, we need collaboration from partners across the health care systems. Stay up to date with CIHI's connected care product developments by [exploring our web pages on connected care](#) and participating in the annual open review process. Together, we can achieve a connected health system and improve care for all Canadians.



Data stewardship



Forging a new foundation for pan-Canadian health data stewardship: The time is now to share information confidently

From coast to coast to coast, CIHI is leading the drive toward more harmonized approaches to data access, sharing and use that will remove barriers and help improve the health all Canadians. We refer to this approach as “health data stewardship.”

In 2024–2025, we worked to complete a new health data stewardship governance model and supporting framework that would ensure Canada’s health information is shared and used responsibly, securely, transparently and efficiently across and within jurisdictions.

Such a strategy is needed because our various health systems have different policies, legislation and practices around data privacy and sharing. For decades, they have rightly protected such data, yet the resulting risk-averse cultures have stalled progress. Under CIHI’s leadership, we can now forge a new foundation for data sharing grounded in collectively agreed-upon principles.



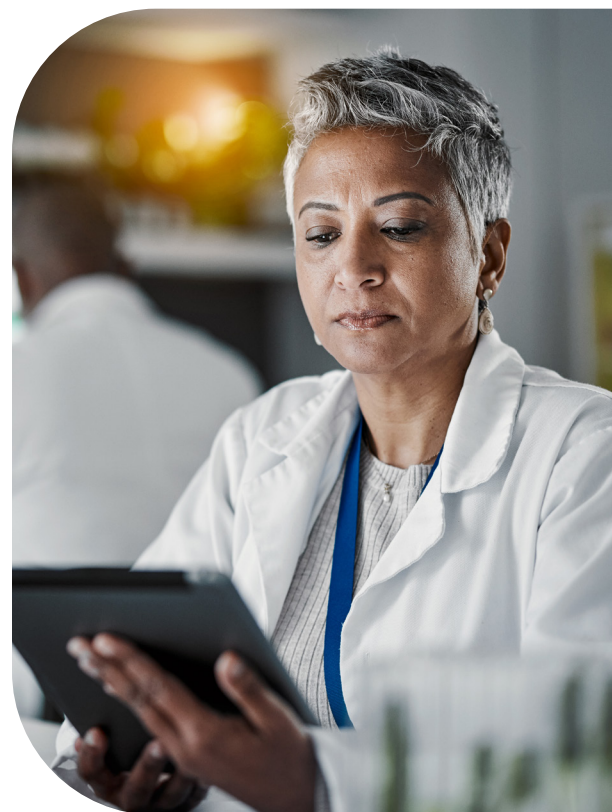


Data travels at the speed of trust

In this transformation, CIHI serves as convenor for all jurisdictions to set priorities and then support long-term health data stewardship efforts.

This takes widespread trust to work. Provinces and territories must trust CIHI's guidance; the public must trust that their personal data is being used appropriately; organizations must trust each other that data is being used safely; and all stakeholders must trust in the quality of the data.

To anchor that trust, CIHI's pan-Canadian health data stewardship framework will pull all important threads together — privacy, ethics, quality, interoperability and more — so that all stakeholders understand what is needed to confidently share and use data. We are also set to release a complementary toolkit that will provide practical resources, best practices, education and support.





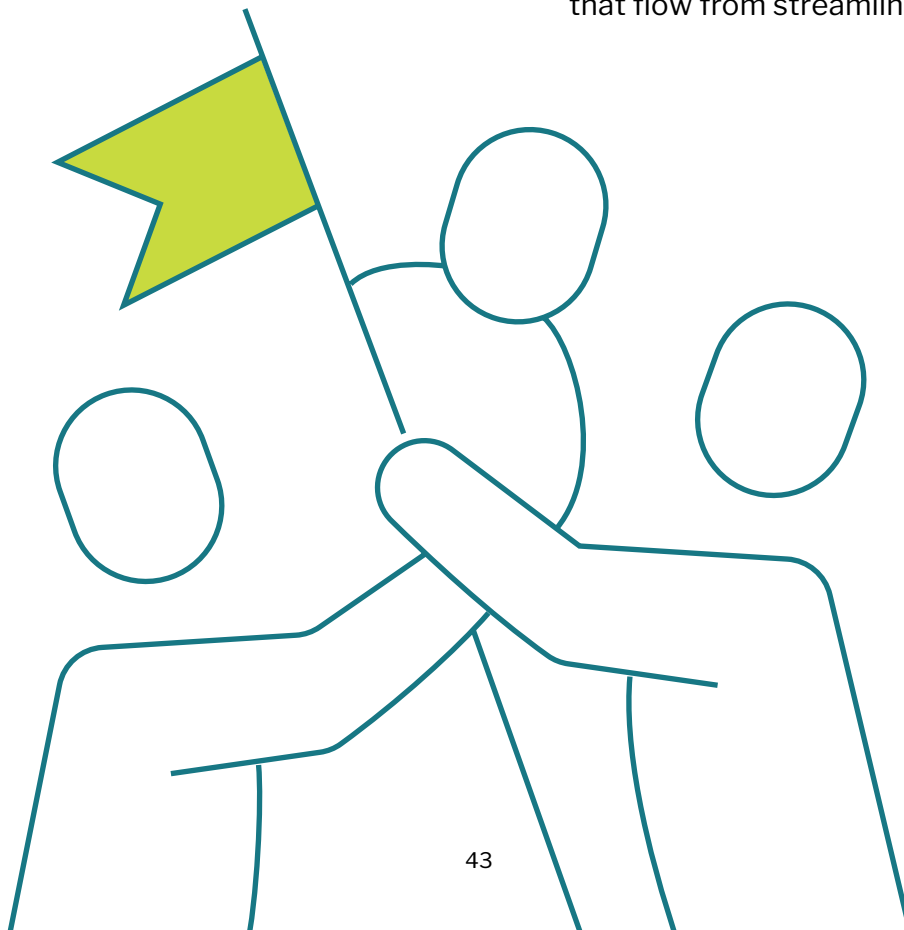
To get to this stage, our teams looked internationally — including at leading data practices in Europe and the U.K. — and collaborated domestically with government partners, Indigenous organizations, Health Data Research Network Canada and Canada Health Infoway, clinicians, patients, the public and digital health experts. In all, everyone involved in the flow of data.





As we review the challenges each jurisdiction faces, we also know that strong health data stewardship requires a change in mindset. It is time to balance privacy with the safe sharing of data, because the benefits are enormous and in our reach:

- Synchronized health data efforts across Canada;
- More data-driven improvements to health systems;
- Improved health system decision-making;
- Higher-quality data through AI;
- Better health outcomes for Canadians; and
- Economic benefits and innovations that flow from streamlined health systems.





Leading the ethical use of health data

While ethics is a long-standing element of research, there has been no common approach to how it helps guide the use of data by our health systems. That is, how health data is captured, managed, analyzed and used to assess health system performance, inform policies, deliver services and allocate resources.

CIHI's new [health data ethics framework](#) is designed to fill this gap, to help us understand, evaluate and address ethical questions around health data and how findings are shared. Ultimately, it is the responsibility of everyone who touches data — providers, data stewards, data analysts, policy-makers and even the media — to ensure it is treated appropriately.

Our framework supports the responsible use of data, describes key ethical principles and covers such important areas as Indigenous data sovereignty and the implications of AI and predictive modelling.

For CIHI, where integrity, privacy and responsibility are always top of mind, this resource formalizes an organization-wide commitment to ethical practices and good health data stewardship. It also informs how our partners and stakeholders can approach data ethics in their organizations — so that we can be better together at both leveraging and protecting health data.

Artificial intelligence



New answers to new questions: CIHI sets a course toward responsible AI

Across Canada, institutions and agencies in the service of health care are embracing the promise of big data and pushing envelopes to test the possibilities of benefiting from AI.

CIHI exists to provide high-quality data that health systems can use to make informed decisions that will improve the health of Canadians. AI presents a huge opportunity to reshape how data is collected, analyzed and disseminated — elevating efficiency, quality and timeliness.

Amidst a remarkable pace of change, we will harness AI so that our partners in the health systems can ask specific questions and receive answers in a timely manner. For ministry officials, care providers, researchers and patients themselves, we have a vision of providing access to live, interactive data so decisions that impact the health of Canadians can be made without delay.

As Canada's health care data custodian, CIHI is positioned to play a key role in this transformation. With such sensitive and significant information in our hands, it is imperative that our systems and processes have the strongest guardrails so that we do this right.





Foundational elements first

Taking AI in the right direction requires us to be as adaptable and flexible as necessary. It will be integrated into CIHI's functions and scaled out, and it must be tied purposefully to outcomes.

A detailed plan is in the works to embed AI sustainably across our data-driven organization. In 2024–2025, the team focused on developing principles around adopting responsible AI, creating a framework for making this transformation responsibly, and upskilling all our staff in this area — foundational elements for our AI journey.

The University of Waterloo is recognized for its expertise in AI and its use in health care, so we partnered with it to design a curriculum that would give all our teams foundational knowledge in this space. Through new eLearning modules, CIHI staff will not only learn how we will use AI and where we must be careful, but also how they can safely use it in their personal and professional lives.

With so much at stake given what health care data can influence, we are expending maximum effort to create responsible AI solutions. First considerations are privacy and security — always top of mind in any new element of our operations.





Then, we face the need for a solution that must be trusted. We must be particularly judicious in our strategy, because AI models will be trained on data that we feed them. Thus any CIHI-developed AI model to analyze data must be free from bias in all elements so any final analytics — and recommendations tied to those results — are not skewed in any way.

Meanwhile, responsible AI must include transparency around how AI is applied to the data. We will need to design models that are explainable and can yield trusted and accurate outcomes. Key principles, policies and governance structures are being developed to guide us on our future practices in this area.

Looking ahead, as we advance our AI strategy and roadmap, we will regularly monitor and learn from other practitioners and the industry and make the necessary adjustments to our plan. We will continue to leverage this capability to our ongoing transformation efforts and to also enable our partners in the Canadian health care ecosystem through collaboration and sharing of experiences. A very exciting time ahead!

Nasir Kenea, Vice President and Chief Information Officer, CIHI



Ask an Expert



How is CIHI using AI to strengthen the data foundation?

As a data organization, we can leverage AI the right way to transform our own processes, revamp how users access information and, as a result, support high-quality care and better patient outcomes.

In the data life cycle, we receive information, enrich and process it, and then make it available in reports, data visualizations and secure portals. Automating this cycle through AI will reduce friction and help uncover a greater variety of insights more quickly than ever before.

Responsible AI solutions will empower institutions to send deeper levels of data faster and will enable CIHI teams to perform analytics and quality assurance in a vastly more efficient and reliable fashion. AI will then transform how we showcase findings so users can engage with and experience data in ways that identify gaps in health as well as potential answers with more immediate clarity.

Nasir Kenea

Vice President and Chief
Information Officer, CIHI



How is CIHI
using AI to
strengthen the
data foundation?



Are there pan-Canadian standards for gender, sex and sexual orientation?

This is an important area in helping understand the differences in how Canadians with various identities experience our health care systems. CIHI led a nationwide scan in 2024 to uncover what level of data the provinces and territories were collecting on this front, and the major gaps we found revealed an urgent need for pan-Canadian standards.

We are fortunate that our partners at the B.C. Ministry of Health are ahead of the curve and have built their own gender, sex and sexual orientation standard. Our teams are now expanding on B.C.'s terrific work by first adopting that B.C. standard and collaborating with a wide range of stakeholders to transform its lens to become pan-Canadian.

This effort will have a tremendous impact on Canadians, because there hasn't been any standardized way to collect this information. Yet we know that not only do men and women connect with their system differently, but that transgender, intersex and non-binary Canadians have especially unique — and often difficult — experiences.

In filling a significant gap, our standard will ensure that data is consistent and reliable as we continue forging safe, equitable health care systems for all.

Shannon O'Connor

Manager, Data Standards, CIHI



Does CIHI have pan-Canadian standards for **gender, sex and sexual orientation?** ”





Case studies

Using data analysis to bridge gaps in dementia care

Challenge

As Canada's population ages, the number of people living with dementia is rising. With nearly 1 million people expected to be living with dementia by 2030, health systems face complex care needs that risk impacting the quality of care provided.

There has been limited understanding of the needs of Canadians who are living with dementia and of their caregivers — understanding both is vital to building effective care strategies as the disease progresses. The Public Health Agency of Canada (PHAC) sought data and metrics that could fill these knowledge gaps and identify best practices for dementia care that could be widely adopted.

Solution

We needed to establish new metrics that would better demonstrate the health complexities and trajectories of people living with dementia. CIHI worked to calculate 4 metrics that had been recently reported to Parliament: depression, pain, social isolation and caregiver distress.

CIHI then released a report in spring 2024, funded by PHAC, that showed great diversity among people who live with dementia and experience a variety of health care trajectories. People were followed for 5 years from their initial diagnosis through home and long-term care settings — and they had unique journeys based on their care needs and the availability of community and family supports.



Impact

This data-driven work demonstrates the genuine experiences of people living with dementia, which can inform what services and policies could flexibly support the distinct needs of patients and caregivers.

In identifying trends and gaps in care, CIHI has provided stronger evidence to support decisions by PHAC and the Ministerial Advisory Board on Dementia and to sharpen Canada's national dementia strategy.

CIHI is part of a knowledge-sharing effort to bring new insights to the Federal, Provincial, Territorial Coordinating Committee on Dementia; the Australian Institute of Health and Welfare; 2024 conferences hosted by the Canadian Public Health Association, Canadian Association of Health Services and Policy Research, and Canadian Association on Gerontology; and the North American Conference on Integrated Care.

Bridging the gaps in dementia care through collaborative surveillance and data analysis

[Back to Case studies: Working together to make an impact in health care](#)

In-depth data surveillance and analysis on care and supports for people living with dementia and caregivers uphold the vision and objectives of Canada's national dementia strategy





Using a digital tool to improve home care in P.E.I.

Challenge

Our partners at Health PEI needed a modern tool that could help them gauge their aging population's care needs and conceive the right home care strategies. Their existing screening tool was insufficient to provide the kind of unbiased, evidence-based assessment needed to develop care plans.

Health PEI needed an efficient, integrated and user-friendly system that would identify the type of home care services people needed, while ensuring that they maintained their quality of life. This digital case management solution also needed to help convert assessment results into personalized, actionable care plans.

Solution

interRAI is the gold standard in the area of home care assessment. We believe the interRAI Home Care (HC) assessment is the most reliable, person-centred tool capable of informing comprehensive care and service planning — and, as such, CIHI used it to transform Health PEI's system.

The interRAI HC, which is on a cloud-based platform that is accessible anywhere, assesses someone's function and quality of life in relation to their needs, strengths and preferences. Over time, it generates an outcome-based assessment of how people with varying care needs respond to home care services. Its evidence-based algorithms then deliver real-time information and recommendations to customize care plans for each Islander.



Impact

By implementing the interRAI HC, CIHI helped Health PEI get reliable assessment results and, for the first time, integrate them directly into care plans.

The assessment tool reduces the burden on clinicians, who can access and share client information quickly and efficiently. And it helps officials determine who needs what types of home care services, and who should receive long-term care and supports to enhance their quality of life — critical aspects of health care funding. 1 Islander, who was previously assessed for long-term care, was able to qualify for home care after his interRAI assessment — a life-changing difference.

Health PEI has improved its system planning and benchmarking capabilities while helping care teams collaborate more seamlessly and ensuring that service and clinical plans are tied directly to each person's functioning, cognitive and mental health, and quality of life.

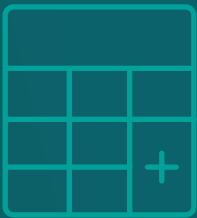
Digital assessment tool for “life-altering good” home care plans

[← Back to Case studies: Working together to make an impact in health care](#)

Health PEI implements a new digital assessment tool to provide better home care assessments and care plans for Islanders



By the numbers



Webinars



Introducing the CACDI and Navigating the Common Data Architecture of the Pan-Canadian Health Data Content Framework | October 1, 2024

430 registrants

New CIHI Indicators — Visits to the Emergency Department for Conditions That Could Be Managed in Primary Care: In Person and Virtual | November 7, 2024

229 registrants

Top 3 downloads



1 Health Workforce in Canada, 2022 — Quick Stats (XLSX)

2 Wait Times for Priority Procedures in Canada — Data Tables (XLSX)

3 Summary Statistics on Organ Transplants, Wait-Lists and Donors, 2023 (XLSX)



Help Desk requests

External emails received
through help@cihi.ca

10,308



Website visits

1.8 million



Podcast streams

33,077



Media mentions

1,000+

Social media

Total followers



129,555

Followers gained in 2024–2025



10,298

Growth rate



8.6%

Our people

CIHI Board member
Dr. Alexandra Greenhill



Dr. Alexandra Greenhill has spent her career using innovation and technology to improve the lives of patients and the ability of physicians to care for them. She is a physician leader, TEDx speaker, expert on AI in medicine, entrepreneur and recipient of the Queen Elizabeth II Diamond Jubilee Medal of Service. She joined CIHI's Board in 2020 and is chair of its Transformation Committee.



Patient care is far more than sticking to general guidelines or devising plans based on taking measurements in the clinic. There is so much to measure that is important, and this is where true data transformations will occur — and allow us to truly understand each patient.

We know that a collection of connected factors keeps a patient well: health care, nutrition, lifestyle, environment, emotional support, informational support, even their own mindset.

We know the health care “system” is in fact a collection of silos: primary care, specialists, hospitals, insurers, pharmaceutical companies, home care, rehab facilities, community-level care, even access to social support.

And we know the reasons for illness extend far beyond the actual delivery of health care: biology, genetics, income, education, employment, housing, sense of community, systemic racism, social support, environmental factors and many more nuanced elements.

Across Canada, there is a tremendous opportunity to spark new collaborations and broaden the scope of data we collect to ensure it serves the genuine needs of patients, clinicians and, ultimately, decision-makers. The more data sources, the more meaning we can derive around those diverse factors that keep people well, that constitute the health care “system” and that threaten people’s health.

We must reach a place where everyone can collect and analyze data. Take this true example from Vancouver: a high school student took open source data to study why there was a dramatic increase in bicycling in the city. They were able to confirm no correlation with availability or gas prices or congestion — and found that new bike lanes were the primary spark.

Imagine what other insights are already available. There is so much to capture and measure that will allow for individualized prevention and treatment plans to be developed earlier than ever before. The way forward must unite both public and private data providers for the first time, must allow for patient feedback at all stages, and must be inspired by Indigenous views of the world and the seventh-generation principle.

CIHI is the trusted organization to lead this transformation, one that is far more complex than acquiring statistics. We can serve as a credible convenor and independent broker of the right stakeholders while also ensuring that data sets are of high quality. This is our opportunity to lead the world and do what others haven’t.

Dr. Alexandra Greenhill

CIHI Board member



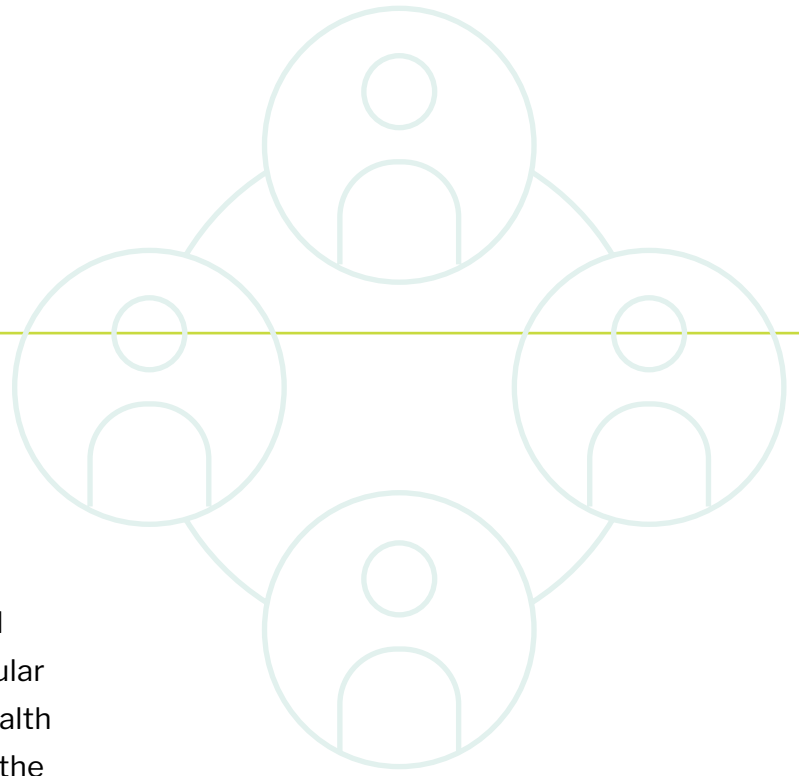


As CIHI's Indigenous health data steward, my work is centred around leading, upholding and strengthening CIHI's approach to respecting First Nations, Inuit and Métis (FNIM) data sovereignty. This role fosters a dynamic working environment as I provide leadership and guidance internally on Indigenous data governance and on advancing cultural safety and humility, while building and maintaining relationships with external organizations and FNIM partners. I find guidance in navigating the duality of this role by drawing from my Mi'gmaq upbringing and the Two-Eyed Seeing Framework (Etuaptmumk), a transformative power that blends Indigenous ways of knowing and Western perspectives while acknowledging their differences and complementarities. I find this work deeply meaningful, as it allows me to contribute to the self-determination of FNIM communities while remaining grounded in my cultural values.



Alex Allard

Program Consultant
Indigenous Health

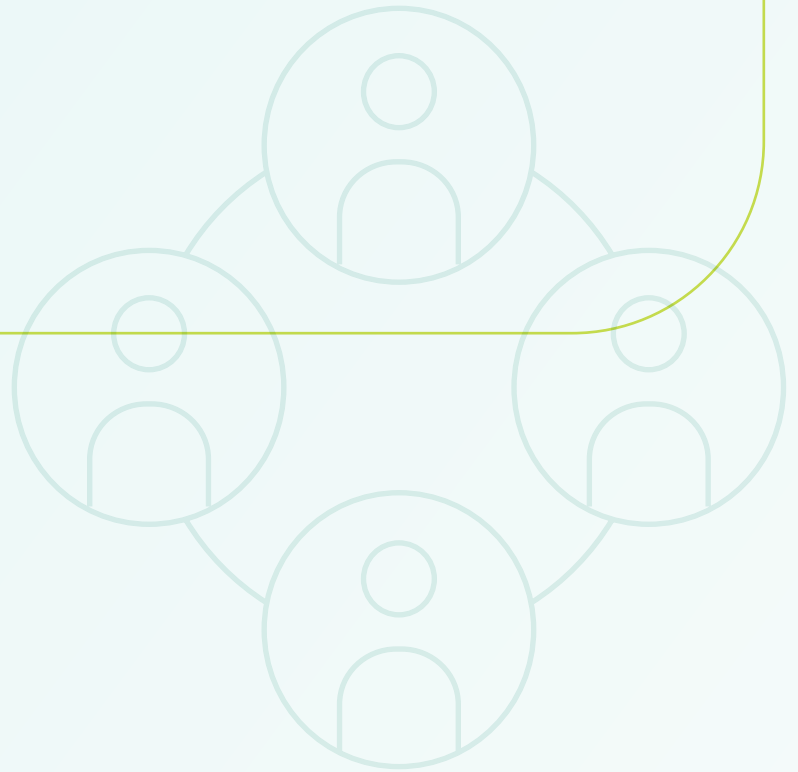


I'm a program lead in the Quebec regional office, and I love working at CIHI in particular because my colleagues — who bring a wealth of knowledge and experience — all share the same values of caring, kindness, collaboration, professionalism and excellence. My role in the Quebec regional office involves working with our stakeholders and providing support for the use of our products to meet their health information needs. It's a privilege to collaborate with health care decision-makers and managers, supporting them in establishing indicators or collecting relevant information, whether to feed into a strategic plan, evaluate programs, support work to establish a new health care policy or improve certain aspects of health system performance.

”

Marie-Noëlle Rondeau

Program Lead
Client Affairs — Quebec

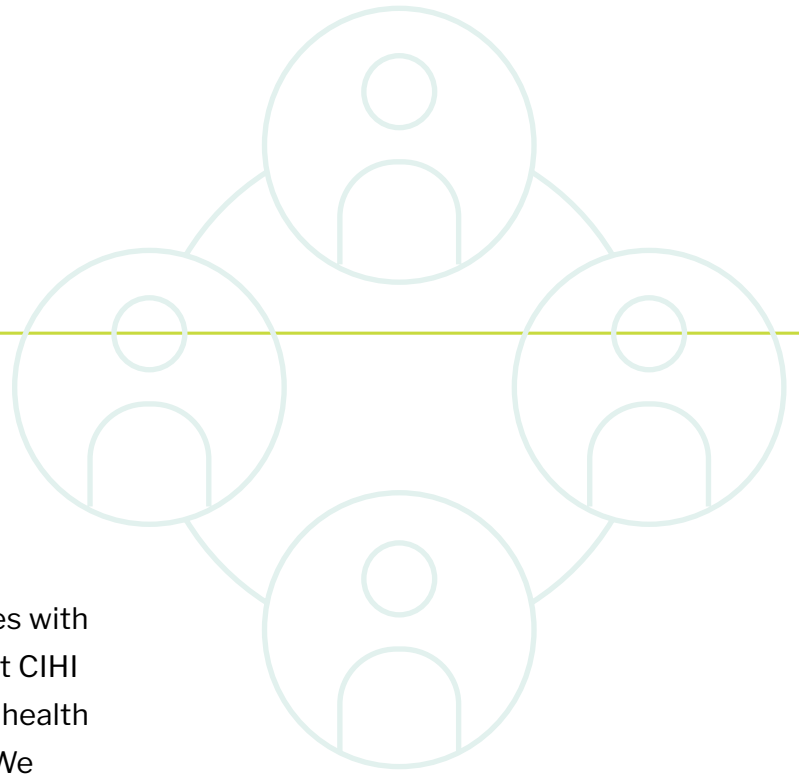


Throughout my time at CIHI, I've worked in a variety of roles, including data analysis and reporting, master data standards and now corporate products. Each position has given me a broader understanding of CIHI and how our work supports health care systems across the country. My current role as a business systems analyst has been a valuable and rewarding step. I collaborate with internal stakeholders to understand their needs and help deliver effective IT solutions. What I value most about CIHI is its people and purpose. The culture is supportive, collaborative and focused on continuous learning. I feel fortunate to be part of an organization where the mission truly matters — using data to improve the health of Canadians. Looking ahead, it's an exciting time to see how CIHI will use technology to strengthen its role in building informed health care systems for Canadians.



Kimberly Ross

Business Systems Analyst
Corporate Products

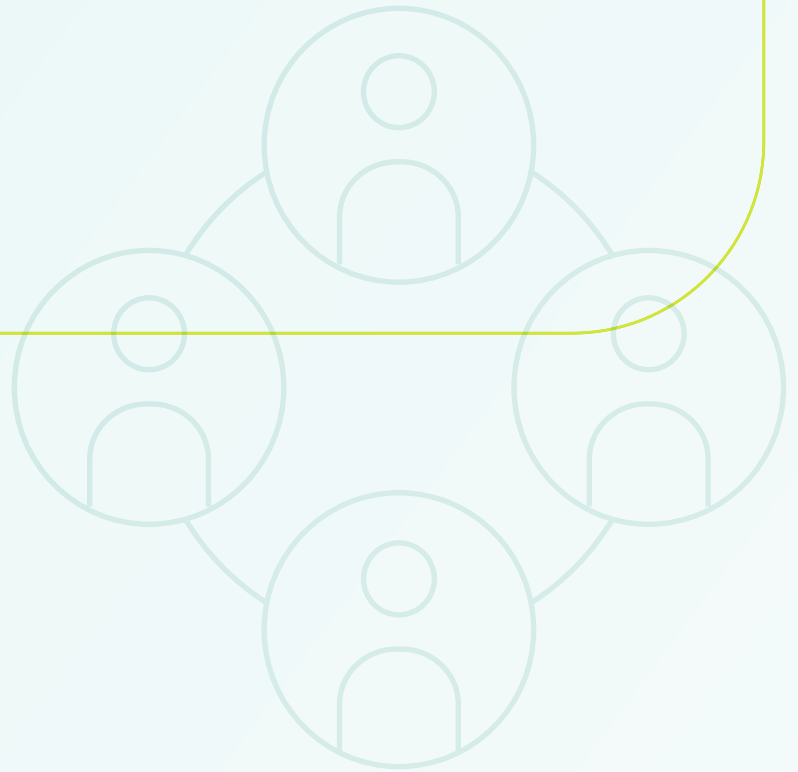


I oversee a talented team that collaborates with health system leaders to use the data that CIHI stewards to shed new light on the critical health system and population issues they face. We connect the dots across vast and diverse data sets to craft clear, practical and impactful data stories that support understanding and action. Currently, our team is developing the Health System Context Series to serve as a backdrop to CIHI's work, supporting interpretations and comparisons across provinces and territories. Through close collaboration across CIHI, we're bringing together a variety of health system metrics that help depict the landscape of health service delivery across Canada. This work builds on the strong foundation of CIHI's data sets and metrics, developed through years of dedicated effort across the organization. There's a renewed sense of energy and direction at CIHI, and I'm proud to contribute to that momentum.



Steve Atkinson

Director
Local Analytics



After working with health care data for many years, joining CIHI gave me an opportunity to work with data standards that help improve Canada's health care systems.

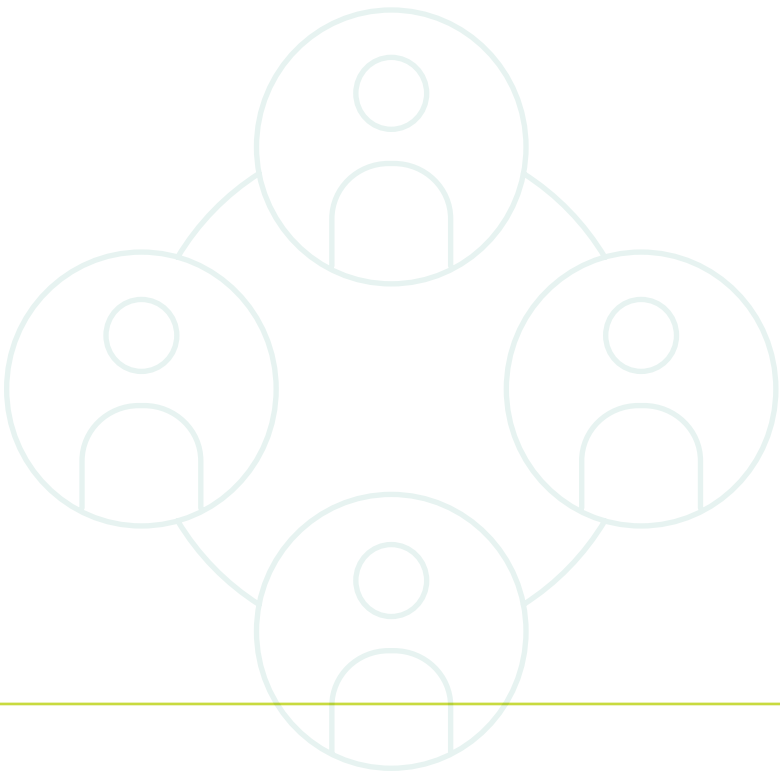
The Management Information System (MIS) Standards is a set of national standards that enables organizations to report financial and statistical data related to health services provided.

Working as part of a team that ensures the MIS Standards remains current, I have come to value the importance of a collaborative team that respects each other's opinions to ensure we have the best product for our stakeholders.



Sylvie Nicholson

Program Specialist
Financial Standards
and Information



Partnering with CIHI has been a highlight of my patient advisor journey. Collaboration, responsiveness and teamwork define my experience. I’ve been involved with several teams at CIHI and my patient insights are deeply valued. Patient advisors contribute to presentations, fact sheets and analyses. Our perspectives bring context to CIHI’s work, as the understandings and decisions gleaned from data collected by CIHI affect all Canadians. As advisors at CIHI, our voice is seen as complementary — and increasingly essential — to other health system stakeholders. Our role continues to grow!



Susan Rich
Patient Advisor



Advancing EDI at CIHI

At CIHI, we've worked hard to build safer, more welcoming spaces where employees can learn and grow together, and where our commitment to equity, diversity and inclusion (EDI) is lived through our day-to-day practices and integrated throughout our organization.

Here are just some of the ways we've helped foster a culture where employees are comfortable being their true selves at work:

- Hosting a series of Telling Our Stories panel sessions to amplify voices and share perspectives on universal themes;
- Providing monthly recognition of significant dates;
- Sharing resources and tools; and
- Engaging employees at events to celebrate National Indigenous History Month, National AccessAbility Week, Pride Month, Black History Month, Lunar New Year, Ramadan and Eid.

Our EDI strategy and workplan provide a shared direction and commitment through 3 goals: to build capacity through learning, to share accountability and to support workforce diversity through inclusive policies and practices.

That commitment extends to our Board of Directors through its orientation process for members, an enhanced diversity profile that supports a membership that draws on the skills and perspectives of a range of leaders, and a public-facing commitment to foster an EDI-driven governance environment while addressing inequities in health systems. CIHI's Board is committed to advancing its own EDI capacity and has featured EDI at Board discussions.

As we work to foster equitable health care systems for all, CIHI teams also ensure that the principles of EDI are threaded throughout our organization and in our collaborations. Some highlights in 2024–2025 include the following:

- We engaged in external EDI-focused communities of practice and working groups to share learnings and contribute to equity projects. These included collaborating on the Health Data Research Network's IDEA strategy, contributing to equity projects with Healthcare Excellence Canada and participating in EDI discussions with our pan-Canadian health organization partners.
- Our EDI Committee expanded its membership and now features 3 subcommittees focused on capacity-building, engagement and events to deepen inclusion and belonging efforts across CIHI. A focus this past year has been to deepen involvement from a broader range of equity-deserving communities in our workplace.
- Around our internal policies, we used an EDI lens to broaden the scope of what constitutes “special leave” and updated CIHI's approach to procurement. Our respectful workplace and recruitment policies have also been updated with an EDI lens, and a decision-support tool is available to assist teams in applying that lens.
- We finalized metrics on things like demographics, workplace experience, policies and practices, culture and environment, and training and development, and we developed a dashboard to display them.
- More than 800 employees engaged in an online, multi-part EDI foundations course. EDI resources are also shared with new employees during orientation and made available in CIHI's virtual learning library.
- We continue to build EDI awareness among all employees through a dedicated learning path. This includes a workshop series for leaders on intercultural communications, psychological safety in the workplace and ways to address systemic racism.





Our leadership and governance

Board of Directors as of March 31, 2025

Dr. Vivek Goel (Chair)

President and Vice-Chancellor
University of Waterloo

Canada at large

Dr. Verna Yiu (Vice Chair)

Provost and Vice-President (Academic)
University of Alberta

Dr. Alexandra T. Greenhill

Founder, CEO and Chief Medical Officer
Careteam Technologies Inc.

Dr. Janet Tootoosis

Family Physician
Vice Dean, Indigenous Health
College of Medicine
University of Saskatchewan

Region 1 • British Columbia

Dr. Maureen E. O'Donnell

Executive Vice President
Clinical Policy, Planning and Partnerships
Provincial Health Services Authority
(Non-government)

Dr. Ian Rongve

Assistant Deputy Minister
Strategic Innovation Division
British Columbia Ministry of Health
(Government)

Region 2 • Prairies

Ms. Charlene McBrien-Morrison

CEO
Health Quality Council of Alberta
(Non-government)

Region 3 • Ontario

Mr. Karim Mamdani

President and CEO
Ontario Shores Centre
for Mental Health Sciences
(Non-government)

Dr. Michael Hillmer

Assistant Deputy Minister
Digital and Analytics Strategy
Ontario Ministry of Health
(Government)

Region 4 • Quebec

Dr. Jean-François Éthier

Professor, Faculty of Medicine
and Health Sciences
Université de Sherbrooke
(Non-government)

Mr. Marc-Nicolas Kobrynsky

Assistant Deputy Minister
Strategic Planning and Performance
Measurement Branch
Ministère de la Santé et des
Services sociaux du Québec
(Government)

Region 5 • Atlantic

Mr. Martin Haynes

Chief Strategy Officer
Medavie
(Non-government)

Mr. Eric Beaulieu

Deputy Minister
New Brunswick Department of Health
(Government)

Region 6 • Territories

Ms. Jo-Anne Cecchetto

Deputy Minister
Northwest Territories Department
of Health and Social Services
(Government)

Health Canada

Mr. Greg Orencsak

Deputy Minister
Health Canada

Statistics Canada

Ms. Josée Bégin

Assistant Chief Statistician
Social, Health and Labour Statistics Field
Statistics Canada

Board committees as of March 31, 2025

Finance and Audit Committee

Members

Karim Mamdani (Chair)
Eric Beaulieu
Josée Bégin
Martin Haynes
Greg Orencsak

Meetings

June 6, 2024
November 18, 2024
February 27, 2025

Governance and Privacy Committee

Members

Verna Yiu (Chair)
Jo-Anne Cecchetto
Jean-François Éthier
Michael Hillmer
Marc-Nicolas Kobrynsky
Ian Rongve
Janet Tootoosis

Meetings

June 4, 2024
November 8, 2024
February 20, 2025

Human Resources Committee

Members

Vivek Goel (Chair)
Alexandra Greenhill
Charlene McBrien-Morrison
Maureen O'Donnell
Verna Yiu

Meetings

November 7, 2024
March 4, 2025

Senior management as of March 31, 2025

Dr. Anderson Chuck

President and CEO

Neala Barton

Vice President

Communications and Client Experience

Brent Diverty

Vice President

Data Strategies and Statistics

Steven Haddad

Executive Director

Provincial and Territorial Relations

Caroline Heick

Vice President

Corporate Services

Nasir Kenea

Vice President and Chief Information Officer

Information Technology and Services

Antony Kent

Executive Director

Finance

Georgina MacDonald

Vice President

Western Canada

Silvia Marabeti

Executive Director

People and Workplace Operations

Kathleen Morris

Vice President

Research and Analysis

Stephen O'Reilly

Executive Director

Strategic Partnerships and Engagement

Nilesh Shastri

Chief Information Security Officer

Chief Information Security Office

Rhonda Wing

Executive Director, Chief Privacy Officer
and General Counsel

Privacy and Legal Services

Steve Atkinson

Director

Western Office and Local Analytics

Herbet Brasileiro

Director

Product Delivery

Ann Chapman

Director

Spending and Primary Care

Cheryl Chui

Director

Health System Analytics

Natalie Damiano

Director
Health Workforce Information

Mélanie Josée Davidson

Director
Health System Performance

Shez Daya

Director
Digital Solutions

Keith Denny

Director
Population and Indigenous Health,
and Classifications and Terminologies

Joanna Filion

Director
Marketing and Communications

Nicholas Gnidziejko

Director
Specialized Care

Matthew Godycki

Director
Architecture and Standards

Cheryl Gula

Director
Thematic Priorities

Jeffrey Hatcher

Director
Advanced Analytics

Tracy Johnson

Director
Pharmaceuticals

Maureen Kelly

Director
Data Governance

Arif Khan

Director
Technology Infrastructure

Connie Paris

Director
Strategy and Operations

Chantal Poirier

Director
Financial Accounting and
Business Operations

Dora Silva Alves

Director
Education, Design and Content Delivery

Juliana Wu

Director
Acute and Ambulatory Care
Information Services

Risk management

Audit Program



CIHI maintains an Audit Program that serves to

- Evaluate the extent to which operations are compliant with applicable administrative policies, procedures and government regulations;
- Assess the overall effectiveness of controls and processes currently in place; and
- Identify opportunities for improvement.

In 2024–2025, activities included the following:

Information security audits/evaluations

- Penetration testing and vulnerability assessments of the ITS network, server infrastructure and selected applications, including the Access Management System.
- Audit of access rights by staff and consultants to CIHI's networks and IT systems.
- Audit of access rights by staff to health data (in progress).
- ISO/IEC 27001:2022 internal audit, surveillance and transition audit.
- Cybersecurity maturity assessment.

Privacy audit

- Privacy audit of a third-party data recipient to determine compliance with CIHI's Data Request Form and Non-Disclosure/Confidentiality Agreement.

We developed action plans to address the recommended areas for improvement that resulted from these audits.



Risk management activities

CIHI's Strategic Risk Management Program is focused on identifying risks that could impede our ability to achieve our strategic goals and meet our commitment to stakeholders, or that might significantly impact CIHI's business continuity or reputation. Strategic risks are identified and assessed annually by members of the executive team.

The goal of CIHI's Strategic Risk Management Program is to foster reasonable risk-taking based on risk appetite and tolerance. It serves to ensure management excellence, strengthen accountability and improve future performance.

Strategic risk management activities for 2024–2025

CIHI's executive team assessed several key risks based on their likelihood of occurrence and potential impacts for inclusion in the 2024–2025 risk program:

- 1. Current and emerging technology needs** — The risk that CIHI would be unable to maintain existing technologies and keep pace with emerging technologies.
 - CIHI expanded and implemented the comprehensive IT Roadmap Program for its technology and application assets, including technology remediation and the implementation of relevant emerging technologies.
 - Given the developments in AI, CIHI developed a strategy to proactively define its approach to AI and guide how it could be leveraged to better serve internal operational and strategic needs and to meet stakeholder needs. Initial steps to support the adoption of AI are underway, such as collaborating on generative AI and sovereign model development, rolling out an AI upskilling program and developing a framework for responsible AI use.

2. Current and emerging timely data supply — The risk that CIHI would be unable to acquire or gain access to timely data.

- Management of risks to the existing data supply included monitoring completeness, timeliness and essential data quality reporting.
- New data advancement initiatives were implemented, including regular engagement with key stakeholders regarding emerging data needs and new data flows (e.g., in health workforce, pharmaceuticals and system performance).
- Provinces and territories were transitioned to modernized CIHI data systems. For example, data reporting for long-term care and home care was streamlined in most provinces and territories (11 out of 13) by supporting adoption of the Integrated interRAI Reporting System (IRRS), as CIHI moved from 3 systems to 1.
- CIHI modernized its standards, systems and processes, and the timeliness of reporting.

3. Balancing priority needs and expectations of funders and data suppliers — The risk that CIHI would not be able to balance the priority needs and expectations of key stakeholders, including various funders and data suppliers.

- Regular engagement with and participation in stakeholder discussions helped CIHI understand and respond to emerging needs while articulating its strengths, neutrality and value to health systems.
- The Data Standards and Data Advancement strategies were aligned with federal, provincial and territorial priorities and capacity.
- The strategic plan was supported by strengthening the analytic gating process, enhancing quarterly planning and reporting, and implementing deliberate approaches to evergreening of products.
- A survey component was launched to support understanding of how key stakeholders view CIHI and interact with our products to support their roles. Survey data will support CIHI's Performance Management Program.
- A new stakeholder engagement framework and government relations strategy were implemented.

4. Susceptibility to a major privacy and security breach — The risk that current privacy and security mitigation strategies would not be sufficient to prevent a major privacy or security breach.

- The Privacy and Security Risk Management Program was maintained, including the Privacy and Security Audit Program and ongoing review and assessment.
- Appropriate processes for supplier frameworks and cloud computing were sustained.
- Privacy and security training and awareness were provided to all staff.
- CIHI's Information Security Management System continued to comply with ISO/IEC 27001:2022.
- Outreach efforts were conducted, and the privacy and security landscape was monitored to ensure CIHI responded to emerging risks and evolving privacy needs of stakeholders, regulators and funders.
- The Demonstrable Accountability Program was sustained, including the ongoing participation of champions embedded across the organization.

5. Resources, funding and operational management — The risk that CIHI would not be able to make progress on strategic initiatives and deliver operational commitments due to organizational stresses related to federal funding agreements and the work required to update key analytical tools.

- CIHI worked with Health Canada toward renewal of base funding and incremental funding for specific objectives.
- Strategic initiatives were prioritized to address the most significant needs first, based on stakeholder demands and confirmed levels of funding.
- Budgets were monitored quarterly, and underspent amounts were reallocated to priority strategic investments and pressure points.
- Oversight and decision-making processes supported prioritization related to analytical transition work, ensuring clear communication with stakeholders regarding impacts on products and services.
- Portfolio management and operational planning were enhanced for better enterprise monitoring of progress against commitments.
- A new marketing strategy was adopted to promote CIHI as an employer of choice.

Leading practices



This section provides an overview of our operations and an explanation of our financial results. It should be read along with the financial statements in this annual report.

Who does what

- Management prepares the financial statements and is responsible for the integrity and objectivity of the data in them. This is in accordance with Canadian accounting standards for not-for-profit organizations.
- CIHI designs and maintains internal controls to provide reasonable assurance that the financial information is reliable and timely, that the assets are safeguarded and that the operations are carried out effectively.
- The Board of Directors carries out its financial oversight responsibilities through the Finance and Audit Committee (FAC), which is made up of directors who are not employees of the organization.
- Our external auditors, KPMG LLP, conduct an independent audit in accordance with Canadian generally accepted auditing standards and express an opinion on the financial statements. The auditors meet on a regular basis with management and the FAC and have full and open access to the FAC, with or without the presence of management.
- The FAC reviews the financial statements and recommends their approval by the Board of Directors. For 2024–2025 and previous years, the external auditors have issued unqualified opinions.

Disclaimer

This section includes some forward-looking statements that are based on current assumptions. These statements are subject to known and unknown risks and uncertainties that may cause the organization's actual results to differ materially from those presented here.

Revenue

Annual sources of revenue

Revenue source (\$ millions)	2024–2025 Budget	2024–2025 Actual	2025–2026 Budget
Federal government — Health Information Initiative*	138.1	125.4	132.5
Provincial/territorial governments — Core Plan	21.1	21.1	21.7
Other	6.9	11.9	9.5
Total annual source of revenue	166.1	158.4	163.7

Note

* Reflects annual revenue on a cash basis, adjusted for the carry-forward projects. Therefore, excludes depreciation expense–related revenue. The revenue amount differs from the amount reflected in the audited statements, which is on an accrual basis.

In the annual sources of revenue table above, the amounts reported as Federal government — Health Information Initiative are based on the actual or budgeted revenue, considering the funding approved as well as the approved carry forwards. The results presented for 2024–2025 include a carry forward of \$18.5 million from 2023–2024, and the budgeted funding for 2025–2026 includes a carry forward of \$5.3 million from 2024–2025. Note: \$1.2 million of the 2025–2026 carry forward will be used to complete the public health system performance indicators work that started in 2024–2025.

Funding agreements

CIHI receives most of its funding from the federal government and the provincial/territorial ministries of health.

Federal government

Since 1999, Health Canada has significantly funded the building and maintenance of a comprehensive and integrated national health information system. Funding has come through a series of grants and contribution agreements referred to as the Roadmap Initiative or Health Information Initiative (HII).

In 2023–2024, CIHI entered into a 2-year agreement with Health Canada for 2024–2025 and 2025–2026, which maintains base funding of \$92.7 million and provides the initial tranches of 2 additional waves of funding totalling \$178 million over 4 to 5 years.

In addition to the ongoing base funding of \$92.7 million, CIHI secured funding for the following initiatives for 2024–2025 and 2025–2026:

Initiatives (\$ millions)	2024–2025 Budget	2024–2025 Actual	2025–2026 Budget
A 5-year Working Together funding allocation totalling \$78.0 million between 2023–2024 and 2027–2028; this investment will significantly advance activities in 3 priority areas: interoperability for connected care, new Shared Health Priority indicators, and data advancement to support health human resources and Health Workforce Canada	14.5	10.2	14.5
A 4-year funding envelope of \$100 million between 2024–2025 and 2027–2028; this investment is to advance transformational leadership and governance of health data and modernization of data infrastructure	5.0	5.0	15.0
Continued funding to develop a modernized organ donation and transplantation data and performance reporting system	4.0	4.1	4.2
Continued funding toward the National Strategy for Drugs for Rare Diseases to increase access to, and affordability of, promising and effective drugs for rare diseases for patients across Canada	0.8	0.7	0.8
Continued funding to operationalize the Physician Resource Planning Tool	0.6	0.5	0.0
Funding toward advancing the development of a suite of public health system performance indicators, which will enable consistent measurement of and reporting on the readiness of Canada's public health systems	2.0	0.8	0.0

Provincial/territorial ministries of health

Through bilateral agreements, the provincial/territorial ministries of health continued to fund our Core Plan (a set of products and services provided to the ministries and identified health regions and facilities).

- These agreements provided \$21.1 million in funding in 2024–2025.
- They were renewed in 2023–2024 for 3 years, through 2025–2026. \$21.7 million has been budgeted for 2025–2026, which reflects a 3% increase as outlined in the 3-year agreements.

Other

Other revenue includes contributions for special purpose programs/projects as well as a variety of other revenues.

Initiatives (\$ millions)	2024–2025 Budget	2024–2025 Actual	2025–2026 Budget
Non-Core Service Agreement with the Ontario Ministry of Health for delivery of identified Ontario-specific data and information products and services that will benefit health sector stakeholders and that will provide a source of comparable and actionable data and information, readily available for informed decision-making, health system planning and management.	2.8	2.9	2.6
A 3-year agreement with Canada's Drug Agency to support the Pharmaceuticals Data and Information Roadmap, which has a goal of bringing enhancements to the pharmaceutical data ecosystem. This agreement provides annual funding of \$3.5 million between 2024–2025 and 2026–2027. The 2024–2025 annual funding was not included in the original 2024–2025 budget since the agreement was fully executed on October 21, 2024.	0.0	3.2	3.5
A collaborative effort with the University of British Columbia to support the Strategy for Patient-Oriented Research, a national initiative aimed at improving health outcomes by integrating research findings into patient care and policy. The agreement started in 2019–2020 and will end in 2025–2026.	0.7	0.7	0.3
Sales, interest and miscellaneous revenue.	3.4	5.1	3.1

Expenses

Operating expenses

Operating expenses (\$ millions)*	2024–2025 Budget	2024–2025 Actual	2025–2026 Budget
Salaries, benefits and pension expenses	131.2	116.3	125.9
External and professional services, travel and advisory committee expenses	14.6	19.2	20.1
Occupancy, information technology and other expenses	20.3	20.0	19.1
Total operating expenses	166.1	155.5	165.1

Note

* Reflects operating expenses; therefore, includes amortization of capital assets.

Total operating expenses, 2024–2025: \$155.5 million

These include compensation costs, external and professional services, occupancy costs and information technology costs required to deliver on several key projects undertaken in 2024–2025. Additional information about employee remuneration is provided in the table below. Total remuneration paid to CIHI's Board of Directors was \$19,950.

Occupational category	Salary range (\$)	Taxable benefits (\$)*	Number of employees†
Administration	36,120–59,980	11–157	4
Support	52,920–86,040	2–231	120
Professional/technical	77,390–125,330	3–350	584
Management and senior professional/technical	111,430–222,440	50–604	118
Vice presidents	197,550–265,970	442–669	6
President and CEO	314,520–423,060	515	1

Notes

* Taxable benefits paid include insurance benefits.

† Number of employees as of March 31, 2025.

Revenue and operating expenses, 2024–2025 planned versus actual

- CIHI delivered on several key initiatives during 2024–2025, given the Health Canada priority funding and additional funds received for various initiatives as detailed above in the Funding agreements section.
- The overall underspend of \$12.7 million, including \$6.2 million to be repaid to Health Canada reflected in the actual, was related mostly to the timing of these additional funds, which caused delays in hiring contract staff and external and professional resources, and which resulted in less travel and fewer advisory committee meetings.

Capital investments

Capital investments (\$ millions)	2024–2025 Budget	2024–2025 Actual	2025–2026 Budget
Furniture and office equipment	0.0	0.0	2.4
Computers and telecommunications equipment	0.2	0.3	0.0
Leasehold improvements	0.0	3.0	2.7
Total capital investments	0.2	3.3	5.1

Acquisition of capital assets, 2024–2025: \$3.3 million

- Capital investments for 2024–2025 were mainly due to leasehold improvements related to the relocation of the Ottawa office, which were funded by a tenant improvement allowance.
- Capital investments over the years are based on an ongoing roadmap of planned acquisitions and upgrades to ensure that equipment and software are robust and adequate to meet changing operational demands.
- The amount of capital asset disposals during 2024–2025 was \$0.4 million, related mainly to the disposal of old IT hardware and software no longer in use.

Planned capital assets, 2025–2026: \$5.1 million

- It is anticipated that most planned capital assets will be expenses for leasehold improvements and furniture in relation to the new Ottawa office lease, funded by a tenant improvement allowance.



Financial statements

Independent auditor's report

To the Board of Directors of the Canadian Institute for Health Information

Opinion

We have audited the financial statements of the Canadian Institute for Health Information (CIHI), which comprise:

- the statement of financial position as at March 31, 2025
- the statement of operations for the year then ended
- the statement of changes in net assets for the year then ended
- the statement of cash flows for the year then ended
- and notes to the financial statements, including a summary of significant accounting policies (hereinafter referred to as the “financial statements”).

In our opinion, the accompanying financial statements present fairly, in all material respects, the financial position of CIHI as at March 31, 2025, its results of operations and its cash flows for the year then ended in accordance with Canadian accounting standards for not-for-profit organizations.

Basis for Opinion

We conducted our audit in accordance with Canadian generally accepted auditing standards. Our responsibilities under those standards are further described in the “**Auditor’s Responsibilities for the Audit of the Financial Statements**” section of our auditor’s report.

We are independent of CIHI in accordance with the ethical requirements that are relevant to our audit of the financial statements in Canada and we have fulfilled our other ethical responsibilities in accordance with these requirements.

We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our opinion.

Responsibilities of Management and Those Charged with Governance for the Financial Statements

Management is responsible for the preparation and fair presentation of the financial statements in accordance with Canadian accounting standards for not-for-profit organizations, and for such internal control as management determines is necessary to enable the preparation of financial statements that are free from material misstatement, whether due to fraud or error.

In preparing the financial statements, management is responsible for assessing CIHI’s ability to continue as a going concern, disclosing, as applicable, matters related to going concern and using the going concern basis of accounting unless management either intends to liquidate the CIHI or to cease operations, or has no realistic alternative but to do so.

Those charged with governance are responsible for overseeing CIHI’s financial reporting process.

Auditor's Responsibilities for the Audit of the Financial Statements

Our objectives are to obtain reasonable assurance about whether the financial statements as a whole are free from material misstatement, whether due to fraud or error, and to issue an auditor's report that includes our opinion.

Reasonable assurance is a high level of assurance, but is not a guarantee that an audit conducted in accordance with Canadian generally accepted auditing standards will always detect a material misstatement when it exists.

Misstatements can arise from fraud or error and are considered material if, individually or in the aggregate, they could reasonably be expected to influence the economic decisions of users taken on the basis of these financial statements.

As part of an audit in accordance with Canadian generally accepted auditing standards, we exercise professional judgment and maintain professional skepticism throughout the audit.

We also:

- Identify and assess the risks of material misstatement of the financial statements, whether due to fraud or error, design and perform audit procedures responsive to those risks, and obtain audit evidence that is sufficient and appropriate to provide a basis for our opinion.

The risk of not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control.

- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of CIHI's internal control.

- Evaluate the appropriateness of accounting policies used and the reasonableness of accounting estimates and related disclosures made by management.
- Conclude on the appropriateness of management's use of the going concern basis of accounting and based on the audit evidence obtained, whether a material uncertainty exists related to events or conditions that may cast significant doubt on CIHI's ability to continue as a going concern. If we conclude that a material uncertainty exists, we are required to draw attention in our auditor's report to the related disclosures in the financial statements or, if such disclosures are inadequate, to modify our opinion. Our conclusions are based on the audit evidence obtained up to the date of our auditor's report. However, future events or conditions may cause CIHI to cease to continue as a going concern.
- Evaluate the overall presentation, structure and content of the financial statements, including the disclosures, and whether the financial statements represent the underlying transactions and events in a manner that achieves fair presentation.
- Communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit and significant audit findings, including any significant deficiencies in internal control that we identify during our audit.

The official financial statements have been signed by the auditors (KPMG LLP).

Ottawa, Canada
June 19, 2025

Statement of financial position

As at March 31, 2025, with comparative information for 2024

	Notes	2025	2024
Assets			
Current assets			
Cash and cash equivalents	3	\$23,370,743	\$30,258,219
Accounts receivable	4	5,934,647	2,814,944
Prepaid expenses		5,789,478	4,533,375
Total current assets		35,094,868	37,606,538
Long-term assets			
Capital assets	5	4,447,355	1,635,504
Other assets	6	219,772	150,921
Total long-term assets		4,667,127	1,786,425
Total assets		\$39,761,995	\$39,392,963
Liabilities and net assets			
Current liabilities			
Accounts payable and accrued liabilities	8	\$16,346,318	\$8,159,773
Unearned revenue		623,240	1,265,308
Deferred contributions	9a, 16	11,039,488	21,589,599
Total current liabilities		28,009,046	31,014,680
Long-term liabilities			
Deferred contributions			
Expenses of future periods	9a	219,772	148,131
Capital assets	9b	980,458	880,491
Lease inducements	10	3,950,107	747,049
Total long-term liabilities		5,150,337	1,775,671
Total liabilities		\$33,159,383	\$32,790,351
Net assets			
Invested in capital assets		252,450	379,384
Unrestricted		6,350,162	6,223,228
Total net assets		6,602,612	6,602,612
Commitments	14		
Total liabilities and net assets		\$39,761,995	\$39,392,963

See the accompanying notes to the financial statements.

The official financial statements have been signed by the Board Chair (Vivek Goel) and the Chair of the Finance and Audit Committee (Karim Mamdani).

Statement of operations

Year ended March 31, 2025, with comparative information for 2024

	Notes	2025	2024
Revenue			
Health Information Initiative	9, 16	\$122,507,787	\$119,413,066
Core Plan	11	21,099,625	20,485,072
Funding — other	12	7,203,357	3,634,875
Sales		2,948,879	2,767,237
Other revenue		1,752,798	863,103
Total revenue		155,512,446	147,163,353
Expenses			
Compensation		116,300,103	107,698,659
External and professional services	16	15,462,935	16,907,695
Computers and telecommunications	16	11,968,419	10,963,954
Occupancy		7,986,863	8,009,498
Travel and advisory committee	16	3,031,156	2,774,053
Office supplies and services		762,970	809,494
Total expenses		155,512,446	147,163,353
Excess of revenue over expenses		\$0	\$0

See the accompanying notes to the financial statements.

Statement of changes in net assets

Year ended March 31, 2025, with comparative information for 2024

	Invested in capital assets	Unrestricted	2025 Total	2024 Total
Balance, beginning of year	\$379,384	\$6,223,228	\$6,602,612	\$6,602,612
Excess (deficiency) of revenue over expenses	(141,847)	141,847	0	0
Change in invested in capital assets	14,913	(14,913)	0	0
Balance, end of year	\$252,450	\$6,350,162	\$6,602,612	\$6,602,612

See the accompanying notes to the financial statements.

Statement of cash flows

Year ended March 31, 2025, with comparative information for 2024

	Notes	2025	2024
Cash provided by (used in)			
Operating activities			
Excess of revenue over expenses		\$0	\$0
Items not involving cash			
Amortization of capital assets		498,058	486,450
Amortization of lease inducements		(261,277)	(295,449)
Amortization of deferred contributions — capital assets		(216,488)	(170,178)
Change in non-cash operating working capital	13	3,168,671	4,447,889
Net change in other assets		(68,851)	(22,208)
Net change in deferred contributions		(10,162,015)	7,833,041
Change in leasehold improvement allowances	10	2,978,541	0
Cash provided by (used in) operating activities		(4,063,361)	12,279,545
Investing activities			
Acquisition of capital assets		(3,309,909)	(424,139)
Cash used in investing activities		(3,309,909)	(424,139)
Financing activities			
Lease inducement received	10	485,794	0
Cash provided by financing activities		485,794	0
Increase (decrease) in cash and cash equivalents		(6,887,476)	11,855,406
Cash and cash equivalents, beginning of year		30,258,219	18,402,813
Cash and cash equivalents, end of year		\$23,370,743	\$30,258,219
Represented by			
Cash		\$18,370,743	\$11,258,219
Short-term investments		5,000,000	19,000,000
		\$23,370,743	\$30,258,219
Supplemental information			
Interest received		\$1,760,587	\$799,161

See the accompanying notes to the financial statements.

Notes to financial statements

1. Organization

The Canadian Institute for Health Information (CIHI) is a national not-for-profit organization continued under Section 211 of the *Canada Not for profit Corporations Act*.

CIHI's mandate is to deliver comparable and actionable information to accelerate improvements in health care, health system performance and population health across the continuum.

CIHI is not subject to income taxes under paragraph 149(1)(l) of Canada's *Income Tax Act*.

2. Significant accounting policies

These financial statements have been prepared by management in accordance with the Canadian accounting standards for not-for-profit organizations in Part III of the *CPA Canada Handbook — Accounting*.

These financial statements do not include the assets, liabilities, or operations of Health Workforce Canada (HWC) and the activities undertaken by CIHI on their behalf are disclosed in Note 16.

Significant accounting policies are as follows:

a) Revenue recognition

CIHI follows the deferral method of accounting for contributions for not-for-profit organizations.

Funding contributions are recognized as revenue in the same period as the related expenses are incurred. Amounts approved but not received at the end of the period are recorded as accounts receivable. Excess contributions that require repayment in accordance with the agreement are recorded as accrued liabilities.

Contributions provided for a specific purpose and those restricted by a contractual arrangement are recorded as deferred contributions, and subsequently recognized as revenue in the same period as the related expenses are incurred.

Contributions provided for the purchase of capital assets are recorded as deferred contributions — capital assets, and subsequently recognized as revenue over the same terms and on the same basis as the amortization of the related capital assets.

Funding contributions received where a funding intermediary agreement has been entered into with another organization are netted against the related expenses.

Interest revenue is recorded as period income on the basis of the accrual method.

Restricted investment revenue and investment losses on restricted contributions are debited or credited to the related deferred contributions account and recognized as revenue in the same period as eligible expenses are incurred.

b) Capital assets

Capital assets are recorded at cost and are amortized on a straight-line basis over their estimated useful lives, as follows:

Assets	Useful life
Tangible capital assets	
Computers and telecommunication equipment	5 years
Furniture and equipment	5 to 10 years
Leasehold improvements	Term of lease
Intangible assets	
Computer software	5 years

c) Lease inducements

Lease inducements, consisting of leasehold improvement allowances, free rent and other inducements, are amortized on a straight-line basis over the term of the lease.

d) Foreign currency translation

Revenue and expenses are translated at the exchange rates prevailing on the transaction date. Any resulting foreign exchange gains or losses are charged to miscellaneous income or expenses. Foreign currency monetary assets and liabilities are translated at the prevailing rates of exchange at year end.

e) Use of estimates

The preparation of financial statements requires management to make estimates and assumptions that affect the reported amounts of assets and liabilities and the disclosure of contingent assets and liabilities at the date of the financial statements, as well as the reported amounts of revenue and expenses during the year. Actual results could differ from management's estimates. These estimates are reviewed annually; as adjustments become necessary, they are recognized in the financial statements in the period they become known.

3. Cash and cash equivalents

Cash and cash equivalents are made up of cash and a short-term investment that has an interest rate of 3.35% (2024: a variety) and an original maturity date of 87 days (2024: 90 days or less).

4. Accounts receivable

	2025	2024
Operating	\$2,454,746	\$2,638,705
Leasehold improvement allowance receivable (note 10)	2,978,541	0
Funding — other	501,360	176,239
Total accounts receivable	\$5,934,647	\$2,814,944

Government refunds receivable at the end of the year are \$320,103 (2024: \$382,628).

5. Capital assets

	Cost	Accumulated amortization	2025 Net book value	2024 Net book value
Tangible capital assets				
Computers and telecommunication equipment	\$1,280,904	\$712,214	\$568,690	\$408,089
Furniture and equipment	2,371,952	2,365,158	6,794	9,105
Leasehold improvements	9,956,735	9,072,077	884,658	1,201,254
Construction in progress	2,978,541	0	2,978,541	0
Intangible assets				
Software	5,896,602	5,887,930	8,672	17,056
Total capital assets	\$22,484,734	\$18,037,379	\$4,447,355	\$1,635,504

Cost and accumulated amortization as at March 31, 2024, amounted to \$19,534,539 and \$17,899,035, respectively.

During the year, CIHI disposed of capital assets with a cost of \$359,714 (2024: \$1,383,329) and accumulated amortization of \$359,714 (2024: \$1,383,329).

As at March 31, 2025, capital assets included \$2,978,541 (2024: \$0) of construction in progress and \$170,988 (2024: \$0) in computers and telecommunication equipment related to the new Ottawa office location. These costs are not subject to amortization until the assets are ready for their intended use.

6. Other assets

Other assets consist of rent deposits to landlords for office space as well as prepaid software, equipment support and maintenance expenses.

7. Bank indebtedness

CIHI has a line of credit of \$5,000,000 (2024: \$5,000,000) with a financial institution bearing interest at the prime rate. This credit facility is secured by a general security agreement on all assets with the exception of information systems.

As at March 31, 2025, there are no draws on the line of credit (2024: \$0).

8. Accounts payable and accrued liabilities

Accounts payable and accrued liabilities are operational in nature and include, in particular, an amount of \$6,229,695 (2024: \$0) representing the annual excess contribution received from Health Canada that is repayable under the terms of the Health Information Initiative agreement.

The government remittance payable at the end of the year is \$262,755 (2024: \$247,758).

9. Deferred contributions

a) Expenses of future periods

Since 1999, Health Canada has been significantly funding the building of a comprehensive national health information system and infrastructure to provide Canadians with the information they need to maintain and improve Canada's health systems and the population's health. Health Canada's funding contribution is received annually based on CIHI's capital resource requirements.

Deferred contributions related to expenses of future years represent unspent restricted contributions. The changes for the year in the deferred contributions — expenses of future years are as follows:

	2025	2024
Balance, beginning of year	\$21,737,730	\$14,143,121
Contribution received from Health Canada	118,358,979	127,683,979
Amount recognized as funding revenue	(122,291,299)	(119,242,888)
Amount allocated to Health Workforce Canada under a funding intermediary agreement	0	(608,050)
Contribution repayable to health Canada (note 8)	(6,229,695)	0
Amount transferred to deferred contributions — capital assets	(316,455)	(238,432)
Balance, end of year	11,259,260	21,737,730
Less current portion	11,039,488	21,589,599
Balance, end of year, long-term portion	\$219,772	\$148,131

b) Capital assets

Deferred contributions related to capital assets include the unamortized portions of restricted contributions with which capital assets were purchased.

The changes for the year in the deferred contributions — capital assets balance are as follows:

	2025	2024
Balance, beginning of year	\$880,491	\$812,237
Amount received from Health Information Initiative	316,455	238,432
Amount recognized as funding	(216,488)	(170,178)
Balance, end of year	\$980,458	\$880,491

10. Lease inducements

The lease inducements include the following amounts:

	2025	2024
Leasehold improvement allowances	\$3,214,447	\$375,629
Free rent and other inducements	735,660	371,420
Total lease inducements	\$3,950,107	\$747,049

During the year, \$3,464,335 (2024: \$0) in lease inducements were recorded. This amount includes a leasehold improvement allowance receivable of \$2,978,541 and other inducements of \$477,504 received related to the new Ottawa office. These amounts will be amortized only when the new office is ready for its intended use.

The amortization of leasehold improvement allowances was \$139,723 (2024: \$144,755). The amortization of free rent and other inducements was \$121,554 (2024: \$150,694).

11. Core Plan

The Core Plan revenue relates to a set of health information products and services offered to Canadian health care facilities, regional health authorities and provincial/territorial ministries of health. Provincial/territorial governments have secured CIHI's Core Plan on behalf of all facilities in their jurisdiction.

12. Funding — other

	2025	2024
Provincial/territorial governments	\$3,204,268	\$2,706,187
Other	3,999,089	928,688
Total funding — other	\$7,203,357	\$3,634,875

13. Change in non-cash working capital items

	2025	2024
Accounts receivable	(\$3,119,703)	\$2,463,877
Prepaid expenses	(1,256,103)	339,194
Accounts payable and accrued liabilities	8,186,545	1,675,068
Unearned revenue	(642,068)	(30,250)
	\$3,168,671	\$4,447,889

14. Commitments

CIHI leases office space under different operating leases, which expire on various dates. In addition, CIHI is committed under various agreements with respect to professional contracts and software and equipment maintenance and support. The minimum amounts payable over the next 5 years and thereafter are as follows:

2026	\$21,860,389
2027	13,367,529
2028	11,771,383
2029	8,075,924
2030	4,234,131
2031 and thereafter	44,157,311
	\$103,466,667

15. Financial instruments

Financial instruments are measured at fair value on initial recognition. Subsequent to initial recognition, they are accounted for based on their classification. Cash and cash equivalents as well as investments are measured at fair value. Accounts receivable net of allowance for doubtful accounts and accounts payable and accrued liabilities are carried at amortized cost. Because of the short-term nature of the accounts receivable as well as the accounts payable and accrued liabilities, amortized cost approximates fair value.

It is management's opinion that CIHI is not exposed to significant interest rate, credit, liquidity, current or other price risks arising from the financial instruments.

a) Interest rate risk

Interest rate risk refers to the adverse consequences of interest rate changes on CIHI's cash flows, financial position and investment income.

b) Credit risk

Credit risk relates to the potential that one party to a financial instrument will fail to discharge an obligation and cause the other party to incur financial loss.

Credit risk concentration exists where a significant portion of the portfolio is invested in securities that have similar characteristics or similar variations relating to economic, political or other conditions. CIHI monitors the financial health of its investments on an ongoing basis.

c) Liquidity risk

Liquidity risk is the risk that CIHI will be unable to fulfill its obligations on a timely basis or at a reasonable cost. CIHI manages its liquidity risk by monitoring its operating requirements. CIHI prepares budget and cash forecasts to ensure that it has sufficient funds to fulfill its obligations.

In addition, as disclosed in note 7, CIHI has an available line of credit that is used when sufficient cash flow is not available from operations to cover operating and capital expenditures.

d) Other

Management does not believe that CIHI is exposed to significant current, foreign currency or other price risks.

There have been no significant changes in CIHI's risk exposure from the prior year.

16. Related party transactions

During the year ending March 31, 2024, Health Canada approved funding of \$2.5 million for pan-Canadian health human resources initiatives, including funding to set up a new entity, Health Workforce Canada (HWC). HWC was created as a separate entity to bring together health workforce and health care experts to strengthen health workforce data and planning. HWC was incorporated as a not-for-profit organization without share capital under the *Canada Not-for-Profit Corporations Act* on October 13, 2023. As of March 31, 2024, one of CIHI's Board members sat on HWC's board, having a 25% representation. In addition to advancing CIHI deliverables with this funding, CIHI acted as a funding intermediary for Health Canada to support HWC operations until March 31, 2024. Given the above, CIHI had a significant influence over HWC. For the year ending March 31, 2024, expenses for external and professional services, travel, and computer equipment were processed on behalf of HWC and netted against the related Health Canada funding in the amount of \$608,050.

As of April 1, 2024, CIHI is no longer a funding intermediary for HWC, as HWC has entered into its own funding agreement with Health Canada. During the year ending March 31, 2025, expenses in the amount of \$419,163 for back-office support, external and professional services, travel, and computer equipment were processed and recovered from HWC. These transactions occurred in the normal course of operations and were measured at the exchange amount, which is the amount of consideration agreed to by the related parties.

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